PHISC
Private Healthcare Information Standards Committee

Private Healthcare Information Standards Committee

TECHNICAL SUB-COMMITTEE: CONTENT WORKGROUP

STANDARDISED CLAIM FORM

Version 5.04

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Revision History

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5.03	2016/07/04	Amend field requirements for tooth numbers – see note and field changes on page 41	PHISC Message Standards Subcommittee
5.04	2017/05/25	Add and amend field requirements for optometry claims	PHISC Message Standards Subcommittee

Glossary

Item No.	Name	Description
1	AHPCSA	Allied Health Professions Council of South Africa
2	Approved	Item financial information relating to the amount that the billing practice will be reimbursed, with a specific view of the member's benefits available. This is a function of the Calculated Amounts, the member's available benefits and the Submitted Amounts.
3	BHF	Board of Healthcare Funders
4	Calculated	Item financial information relating to the amount that the billing practice could have charged for a service based on the rules of the medical scheme and / or legislative requirements (PMBs) and / or special authorisations approved by the medical scheme irrespective of the member's available benefits. With Rules of the Medical Scheme could include the medical scheme's reimbursement rate and all / any contractual additions and / or deductions that could apply and special authorisations.
5	CCSA	Complete CPT for South Africa
6	CDL	Chronic Disease List
7	CDT	Current Dental Terminology
8	Claim	Refers to Invoice, Tax Invoice and / or Claim.
9	Claimed Amount	This is the gross less any discounts (note it includes the patient portion)
10	Co-Payment / Surcharge	Co-payment is a variable fee based on a Medical Scheme member's behaviour e.g. amount available in Medical Savings Account (MSA). The supplier / billing practice cannot predict this fee. This is a fixed Rand value or percentage value calculated at script or item level, which is due by the patient at the point of service according to the Scheme's registered rules. The co-pay amount might be deducted from the total cost of the service The co-pay amount will not be refunded from the medical scheme risk benefit but might be paid from the member's savings benefit.
11	Cost	This is the SEP or wholesale price of a NAPPI code
12	CPT	Current Procedural Terminology
13	Daily Dosage	Defined daily dose of NAPPI item consumption.
14	Days Of Therapy (DOT)	This is the number of days that the prescribed or dispensed NAPPI item should be used. It is usually indicated by a number of days. For example 15 capsules are dispensed and the patient should take 3 capsules per day, then the DOT will be 5 days.
15	Drug Overcharge	If the price submitted is higher than the price approved (i.e. SEP + appropriate dispensing, professional or mark-up fee), a drug overcharge will be returned as the difference between the submitted and the approved amount.
16	EDI	Electronic Data Interchange
17	Encounter	A grouping of events – linked to a single Supplier and Person responsible for the account.
18	Event	A single or grouping of incidents
19	Gross	This is the cost of the item plus mark-up and dispense fee
20	HPCSA	Health Professions Council of South Africa
21	ICD10	International Classification of Diseases version 10 (International Statistical Classification of Diseases and Related Health Problems 10th Revision)
22	IP	Intellectual Property
23	Incident	Each line item – every action taken / service rendered
24	Levy / Deductible	Levy is a predictable fee indicated by the supplier / billing practice, based on patients' plan selection. Generally applied to consultations. This is a fixed rand or percentage value calculated at script or item level, which is due by the patient at the point of service according to the Scheme's registered rules. The levy amount might be deducted from the total cost of the service The levy amount will not be refunded from the medical scheme risk benefit but might be paid from the member's savings benefit.
25	NHRPL	National Health Reference Price List
26	OCR	Optical Character Recognition

Item No.	Name	Description
27	Patient Liable Portion	Levy Plus co-payment due by the patient
28	PBM	Pharmaceutical Benefit Management
29	PCNS	Practice Code Numbering System
30	PHISC	Private Health Industry Standards Committee
31	PMA	Practice Management Application
32	PMB	Prescribed Minimum Benefits
33	PPN	Preferred Provider Network.
34	Reference Surcharge	In those instances where a Scheme has adopted a reference pricing model, but where the patient elects to take the higher-priced product prescribed for him/her, the patient is liable for the difference in the Calculated Gross Price of the respective products. The difference is referred to as the Reference Surcharge and is the amount payable by the member to the pharmacy over and above the calculated Levy. This Reference Surcharge does not alter the 'Due by Scheme' amount
35	SACSSP	South African Council for Social Service Professionals
36	SADTC	South African Dental Technician Council
37	SAMA	South African Medical Association
38	SANC	South African Nursing Council
39	SAOA	South African Optometric Association
40	SAOPA	South African Orthotic and Prosthetic Association
41	SAPC	South African Pharmacy Council
42	Savings Surcharge	A Savings Surcharge applies in those instances where the member is liable for an amount, and the amount is deducted from the member's savings benefit. This is an informational field.
43	SEP	Single Exit Price
44	Scheme Liable Portion	This is the claimed amount less the patient liable portion
45	Scheme Surcharge	A Scheme Surcharge applies in those instances where the Scheme has implemented customised status indicators on the benefit file. A Scheme Surcharge also applies in any scenario where the member is liable for any amount as per the Scheme's rules, which includes all co-pay and levy/deductible amounts. The patient is liable for any Scheme Surcharge as calculated according to the Scheme's customised status rule as well as according to the member's current status as provided on the benefit file. Scheme Surcharges can be calculated at item or claim level.
46	Submitted	Item financial information relating the amount that the practice is claiming from the medical scheme for reimbursement for services rendered to members.
47	Supplier	This is the billing practice – the entity that gets paid
48	Transaction	An electronic transaction has been defined as a collection of tightly coupled messages. A Claim transaction includes the Claim Message and the Claim Response Message
49	UN/EDIFACT	United Nations Electronic Data Interchange For Administration, Commerce and Trading
50	WHO	World Health Organisation
51	XML	Extensible Mark-up Language
52	XSD	XML Schema Definition

1. General Information

1.1 Introduction

PHISC established the Standardised Claim Form Committee in 2005 with the objective of defining standards that could address inefficiencies in the claims administration value chain in the South African private healthcare sector arising from inconsistencies in both paper and electronic claim formats.

To this end, the committee actively recruited various parties in the industry to ensure the following:-

- Sufficient involvement and representation from Funders, Providers, PMA Vendors, Switching Intermediaries and other 3rd parties.
- Relevance and sufficient coverage when considering the diversity of software systems and business rules in the industry.
- The change impact of the standard in the industry is reasonable and sufficiently motivated by its expected benefits.
- The standard fairly balances the need for simplicity and low cost of implementation with the needs for strategic flexibility.

Workshops were held as open forum sessions with voluntary participation by interested parties. Formal feedback was provided at the broader PHISC committee meetings held quarterly. At these quarterly meetings the following key principles and assumptions have been reiterated and have served to govern the design of the standard:

- The efficiencies which accrue this standard are dependent on broad based adoption and therefore cannot be viewed as a long term sustainable basis for competitive advantage by any single adopter. However, it is critical that all stakeholders should derive benefit from the standard in order for adoption to be meaningful. As a consequence, PHISC holds the position that it's messaging standards must remain in the public domain for it to continue to be significant driver of industry-wide operational efficiency.

- It is agreed by the PHISC representative that competitive advantage, in an operational sense for adopting organisations, is vested rather in the service / product / benefit designs, partnerships, sourcing arrangements, production processes of each individual adopter than the messaging standard and therefore every attempt has been made to ensure that the messaging standard does not constrain their current and future choices in this regard.

1.2 Structure of the standard

The standard consists of:-

- Standardised Claim Form Content
  At the highest level this part of the specification specifies the information requirements of a conceptual Claim, Amended Claim and Credit Note, a formal vocabulary and the business rules for application purposes.

- Modifier Submission Standard
  Tightly coupled to the Standardised Claim Form Content, this standard defines a framework for categorising modifiers and how each modifier's type should be submitted. It then classifies the current list of modifiers into the four (4) modifier types, Informational, Addition, Reduction and Compound modifiers, and provides extensive application examples.

- Paper Claim / Amended Claim / Credit Note Layout Specification
  Given the formal vocabulary and business rules defined in the Standardised Claim Form Content, this specification provides the templates for generating a standardised paper Claim, Amended Claim and Credit Note.

- XML Claim / Amended Claim / Credit Note / Response Layout Specification
  As with the paper layout, the XML layout also takes direction from the Standardised Claim Form Content and defines how the conceptual Claim should be represented in a standardised XML message for real-time or batch electronic submissions. This specification also defines a response message protocol that allows multiple responses from multiple parties, including processing intermediaries such as health benefit management companies, in the submission channel as well as extensive message tracking, message and batch level envelope structures that allow for message payload encryption. All the data elements documented in the Content specification will be mirrored when a Response message is generated, with the proviso that it was submitted and received parties.

- XML Electronic Admittance Advice Layout
  Given the close relationship between the Claim, the response and the remittance advice, a first draft of the design for the electronic remittance advice was made in the design phase of the claim to ensure compatibility. There are two (2) key strengths of this component: (1) As with the response messages of the claim it provides a framework for consistent and unambiguous claim processing outcomes communication, (2) its ability to carry the unique invoice numbers received in the claim and thereby permit automated reconciliations in the Practice management software, further driving efficiencies in the market.

- Style sheet Transformation
  An XSLT style sheet transformation has been developed by members of the XML Proof of Concept workgroup. This XSLT style sheet will allow Practice Management Application (PMA) vendors to build their systems around the XML specification and at claim submission time, either submit the claim electronically as XML or simply apply the XSLT style sheet. Two (2) versions will be produced: one for continue feed printers like dot matrix and another for single sheet printers like laser printers. This approach is expected to dramatically reduce the cost of adoption for PMA vendors and improve the rate of adoption.

1.3 Purpose of the document

This document should serve as the design framework in respect of the Standard Claim Form message standard. The actual implementation of this initiative could differ from funder to funder and provider to provider.

The objective of this document is to structure the information requirements required on a Standardised Claim Form for both paper and electronic claims. This document describes the requirements for the content of any Claim, the standardisation guidelines for Modifiers as decided by the PHISC Software Technical Sub-committee's Content Workgroup. This document does not describe the paper layout or the electronic XML layout, which has been constructed by the PHISC Software Technical Sub-committee.

The objective of this PHISC Software Technical Sub-committee is to establish standardised claim content, a standardised layout for paper and electronic Claims. By adopting the standardisation there will be higher turnaround by the Medical Schemes for processing and a standardised way across the industry for claiming. This will drive efficiencies and consistency in compliance to legislative requirements.

The Standardised Claim Form will be maintained quarterly by the PHISC Software Technical Sub-committee's Content Workgroup. These standards serve as a reference guide and the application thereof is voluntary and is only applicable to the South African healthcare sector. The Standardised Claim Form is intended for receipt by the Medical Scheme as well as the Patient / Member.

1.4 Intent of sections of document

Due to the diversity in application of business processes across provider disciplines it has been agreed that different claims may need to be created for different market sectors, for example Doctors, Pharmacies and Hospitals. Each claim form will be based on a standard format but the individual provider requirements would be catered for in the detail of the form e.g. Doctors, Pharmacies, Pathology and Hospitals.

Please note that these requirements do not cater for claims submitted by Facility type practices like hospitals and clinics. The standardised information requirements for these claims will be published at a later stage. The facilities that have been excluded from the scope of this document are listed below in Figure 2.

The design of the generic claim form does not embrace clinical assessment requirements and is only intended for reimbursement purposes. Information requirements for clinical purposes will be addressed at later stage. In the case of dispensed medication the claim form does not make provision for broken bulk due to lack of standardisation in the industry and legislative changes. Hence, standards utilised at a specific Medical Scheme level will have to be applied.

The use of diagnosis and procedure coding is dependent on the respective service provider rendering the relevant healthcare service. Hence, multiple coding standards will need to be catered for.

The minimum information requirements for reimbursement purposes is dependent on the type of service rendered, the type healthcare service provider and the Medical Scheme rules as the layout provided is a generic combination of information requirements catering for multiple scenarios. Thus, a subset of these information requirements will result in the minimum claim information requirements to be supplied subject to the type of healthcare service and the respective healthcare service provider rendering such a service.

1.5 Summary of scope

The standard caters for the following:-

- Provider disciplines: GP, Dental, Auxiliaries, Specialists, Pharmacy, Optometry, Pathology and Radiology

- A Claim is defined by the following attributes:-
    - A **single patient**, who is a member or a dependent of a member, of a single Payor e.g. medical scheme
    - Across **multiple dates of service**
    - Treated by **multiple treating providers** in the same Claim and
    - Referred by **multiple referring providers**
    - Who, in turn, make use of the administrative facilities of a **single billing practice**.

- Message types:-
    - Claim
    - Amended Claim
    - Credit Note

- Presentation formats:-
    - Dot matrix and single sheet printer i.e. laser printers printouts
    - Batch or real-time XML electronic submission.

Our sincere thanks to all parties that have contributed in whatever manner to the formulation of these requirements and hope that this assists with increased service delivery and efficiency.

2. Content for the Standardised Claim

2.1 Introduction and General Information

- A Claim is defined as the collated information documented for reimbursement purposes that will be sent to a Payor for reimbursement for a single patient and / or clinical encounter.
- A message sent to a Billing Practice is defined as a Response or a Remittance Advice.
- The principle of the Standardised Claim, whether on paper or electronic, is that a SINGLE Claim represents a SINGLE Billing Practice, SINGLE patient / member, MULTIPLE treating / referring providers, MULTIPLE service dates.

  The decision to cater for a single patient and / or clinical encounter is based on the contract undertaken between a patient and the Billing Practice.
- The originator of the message is responsible for the uniqueness of the Claim / Invoice / Amended Claim / Credit Note on their system by assigning a globally unique Claim number that cannot be repeated over time.
- Whilst this content document might define a particular field as CM (Conditional Mandatory) and the conditions are determined by external factors, these fields will be reflected as Optional (O) fields in the XML document.
- The Content section of this document does not impose the structure that needs to be used when submitting claims. The structures will be discussed in detail for each submission format; paper or electronic.

2.2 Design Principles

- Best practice recommendation: An Amended Claim and / or Credit Note can be submitted at any time and it is not necessary for the Medical Scheme to first receive a Claim and then only the Amended Claim or for the Billing Practice to wait for a Remittance Advice (RA) before submitting a Credit Note.
- Global uniqueness of a Claim is achieved using: Billing Practice PCNS number + Unique Claim Number determined by the Billing Practice.
- Global uniqueness of a Claim Line is achieved using: Billing Practice PCNS number + Unique Claim Number determined by the Billing Practice + Line Item Identifier.
- Malformed XML messages has been defined as:
  The assumption is made that the envelope can be opened. In this instance, it should be indicated on envelope level that the message was malformed. In instances where the envelope cannot be opened, NACs will be handled at the communication level of the TCP/IP stack.
- All message types: PHISC standard excludes any encryption implementation for XML messages.
- It is recommended that the Standardised Claim Form only be printed in English.
- All currency indicators can be omitted from the Standardised Claim Form as it is only applied to South African claims and hence the currency is assumed to be Rands.
- When printing a data code or mosaic, this data code must be printed on the last page of each Claim form.
- The tables used in section 2.3 Standardised Claim Content to describe the data elements that are required to be used have a column called ‘Practice Type Claim Submission’ that is divided into sub-columns specific to certain practice types. In the columns for each practice type, one of following two (2) notations are used:-

  - Mandatory (M)
  - Conditional Mandatory (CM)

  The notations indicate when the data element is required to be included as part of a Claim, an Amended Claim or a Credit Note. In some instances, the information requirement is dependent on a specific condition to be met before becoming Mandatory. These data elements have been marked with a Conditional Mandatory (CM) indicator. In XML there is no provision for CM indicators, therefore in the XML documentation the notation is Optional (O).

2.3 Standardised Claim Content

Please take note that the Practice Type Claims Submission column is divided into sub-columns. These sub-columns are based on specific practice types. The differentiation between the different practice types was deemed necessary where message / printout content differ significantly. The following sub-columns have been used:-

- General Practitioners / Specialists / Auxiliaries
- Pharmacy
- Optometry / Ophthalmology
- Pathology

The following table documents the practice types (excludes sub-discipline) that have been grouped under each Practice Type Claim Submission sub-column:-

Practice Type Claims Submission	Practice Type Prefix	Practice Type Descriptions
General Practitioners &	014 / 015	General Medical Practitioner
Dental Practitioners &	054 / 095 / 096 / 093	General Dental Practitioner / Dental Therapy / Community Dentistry / Dental Technician Laboratory
Specialists & Auxiliaries	010 / 044 / 021 / 012 / 038 / 035 / 019 ‘ 062 / 023 / 024 / 020 / 025 / 016 / 026 / 064 / 028 / 030 / 032 / 033 / 092 / 034 / 036 / 094 / 022 / 017 / 040 / 031 / 018 / 042 / 046 / 050	Anaesthesiology / Cardio-Thoracic Surgery / Cardiology / Diagnostic Radiology / Emergency Medicine Independent Practice Specialist / Gastroenterology / Maxillo-facial and Oral Surgery / Medical Oncology / Neurosurgery / Neurology / Nuclear Medicine / Obstetrics and Gynaecology / Ophthalmology / Orthodontics / Orthopaedics / Otorhinolaryngology (ET) / Paediatrics / Paediatric Cardiology / Periodontics / Physical Medicine / Plastic and Reconstructive Surgery / Prosthodontics / Psychiatry / Pulmonology / Radiation Oncology, Nuclear Medicine or Oncology / Rheumatology / Specialist Physician / Surgery or Paediatric Surgery / Urology / Group Practices
	003 / 091 / 067 / 082 / 075 / 078 / 004 / 090 / 084 / 083 / 069 / 029 / 074 / 087 / 102 / 072 / 103 / 068 / 086 / 085 / 039 / 081 / 088 / 089 / 082	Accredited Blood and Blood Product Couriers / Ambulance Services / Art Therapy / Audiology / Biokinetics / Blood Transfusion Services / Chiropractors / Clinical Services / Dieticians / Hearing Aid Acousticians / Medical Scientists / Occupational Therapy / Orthoptists / Orthotists and Prosthetists / Osteopathy / Physiotherapy / Phytotherapy / Podiatry / Psychology / Psychometry / Radiography / Registered Councellors Registered Nurses / Social Workers / Speech Therapy and Audiology
Pharmacy	060 / 061	Pharmacies / Pharmaco-therapists
Optometry	070 / 071	Optometrists / Optical Dispensers
Pathology	052 / 027 / 098	Pathology / Clinical Haematology / Oral Pathology

Figure 1: Practice Types covered by this version of the Standardised Claim Form document

The following table documents the practice types that have been EXCLUDED from the scope of this document:-

Practice Type Prefix	Practice Type Descriptions
009 / 011 / 013 / 077 / 047 / 079 / 055 / 080 / 057 / 058 / 059 / 056 / 049 / 048 / 076 / 051	Ambulance Services: Advanced / Ambulance Services: Intermediate / Ambulance Services: Basic / Approved Unattached Operating Theatre Units (UOTU) and Day Clinics / Drug and Alcohol Rehabilitation / Hospices / Mental Health Institutions / Nursing Agencies and Home Care Services / Private Hospital Status A / Private Hospital Status B / Private Rehabilitation Hospitals / Provincial Hospitals / Sub-Acute Facilities / Travel Clinics / Unattached Operating Theatres and Day Clinics / Group Practices and Hospitals

Figure 2: Practice Types not covered by this version of the Standardised Claim Form document.

2.3.1 Date and Account Number

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Date Transaction Created	8N Date format CCYYMMDD 5AN HH:MM	Claim Date: Date the message / printout representing the Claim is created at the point of service. This field is NOT used to indicate a unique Claim. Paper: Default Claim date. Electronic: Date and Time Claim sent.	Both	M	M	M	M
Account Number	20AN	Supplier / Billing Practice's account number for that patient's claim. Also referred to as a Control account / Statement / File Number, Patient Number. For Suppliers / Billing Practices that don't use a unique file number and already use unique invoice numbers for each claim created, this unique number can be duplicated as the invoice number in the account number field.	Both	M	NOT Required	M	M

2.3.2 Supplier / Billing Practice Information

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Supplier / Billing Practice PCNS number	18AN	The Supplier is the Billing Practice - the entity that gets paid. Note that the last 7 characters are the Supplier / Billing Practice's sequential PCNS registration number **but that ALL 13-digits of the PCNS number MUST be included on all claims.** A PCNS number of 13-digits must be submitted - without any spaces or other punctuation marks and keeping the leading zeroes intact. Mandatory for COID claim.	Both	M	M	M	M
Supplier/Billing Practice VAT Registration Number	20AN	CM: Include when the Billing Practice / Supplier is VAT registered. Mandatory for COID claim if VAT must be paid by Compensation Commissioner.	Both	CM	CM	CM	CM
Supplier / Billing Practice Name	60AN	This is the Billing Practice's registered Name, not the Trading As name (if applicable). CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims.	Both	CM	CM	CM	CM
Supplier / Billing Practice Address Line 1	25AN	Building CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims. CM: Information must be available to party generating original message i.e. Claim.	Both	CM	CM	CM	CM
Supplier / Billing Practice Address Line 2	25AN	Street Name and Number OR Postal address. Please refer to the note below regarding more information on the submission of postal address details. CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims. CM: Minimum data required regardless of submission method (paper / electronic)	Both	M	M	M	M
Supplier / Billing Practice Address Line 3	25AN	Suburb. CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims. CM: Information must be available to party generating original message i.e. Claim.	Both	CM	CM	CM	CM
Supplier / Billing Practice Address Line 4	25AN	Town. CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims. CM: Minimum data required regardless of submission method (paper / electronic)	Both	M	M	M	M
Supplier / Billing Practice Address Line 5	10AN	Postal Code. CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims. CM: Minimum data required regardless of submission method (paper / electronic)	Both	M	M	M	M

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Supplier / Billing Practice Telephone Country Code	10AN	International standards, need country code CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims. CM: Information must be available to party generating original message i.e. Claim.	Both	CM	CM	CM	CM
Supplier / Billing Practice Telephone Area Code	10AN	Area Code CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims. CM: Information must be available to party generating original message i.e. Claim.	Both	CM	CM	CM	CM
Supplier / Billing Practice Telephone Number	10AN	Telephone Number CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims. CM: Information must be available to party generating original message i.e. Claim.	Both	CM	CM	CM	CM
Supplier / Billing Practice Service Location Identifier	5AN	Each Billing Practice will be required to assign a code which is unique per practice treating location, this means the geographical location i.e. different branches and not the technical location i.e. server name. A code could be used instead of a description of the location to avoid validation problems arising from variations in descriptions. This field allows Funders to design benefit structures for members that is treating location specific. PHISC will not assume responsibility to maintain a list of standard Service Location Identifiers. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM

Note:

- The fields Supplier / Billing Practice Telephone Country Code, Supplier / Billing Practice Telephone Area Code and Supplier / Billing Practice Telephone Number can be concatenated when printed on a paper Claim or submitting electronic claims.

- The fields Supplier / Billing Practice Address 1, 2, 3, 4 and 5 can be concatenated when printed on a paper Claim.

- When a message is generated that needs to contain postal address information, the following example illustrates how information could be reflected: [Postnet_Suite_1234_Private_Bag_X1234]

  The information illustrated in this example could be split into the two Address Line fields (Address Line 1 and Address Line 2) as:
  [Address Line 1]: [Postnet_Suite_1234]
  [Address Line 2]: [Private_Bag_X1234]
  Alternatively, to comply with the minimum data requirements: [Address Line 2]: [Postnet_1234_PBag_X1234]

2.3.3 Medical Scheme Information

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Registration Number	15AN	Use the CMS' List of Registered Medical Schemes in South Africa document's RefNo column as the de facto industry standard for most medical aids. Some medical schemes are not covered by the Medical Schemes Act and therefore do not require registration with the CMS. In order to identify these medical schemes a unique number will be assigned by PHISC. Refer to Note 1, Note 11 and Appendix C for the list of PHISC List of Medical Schemes Excluded from CMS Registration. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Registration Type	5AN	This indicator will identify the source of the Registration number. Refer to Table 11. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Medical Scheme Name	50AN	Medical Scheme Name CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims.	Both	CM	CM	CM	CM
Claim Option	10AN	CM: Claiming arrangement(s) specific to an Invoice and a particular Medical Scheme can be indicated using this field based upon contract / network / reimbursement arrangements between Medical Scheme and third party.	Both	CM	CM	CM	CM
Medical Scheme Option / Plan Reference	10AN	This field describe the registered plans or options of a Medical Scheme with the Council of Medical Schemes (CMS). CM: Where a Medical Scheme requires the Option or Plan reference, the information must be supplied and readily accessible i.e. on the membership cards.	Both	CM	CM	CM	CM

Note:

- None of the information documented in section 2.3.3 Billing Event: Medical Scheme Information is required for COID claims.
- Note 1: List of Registered Medical Schemes in South Africa

  The latest List of Registered Medical Schemes in South Africa can be downloaded from the Council of Medical Schemes' web site using the following URL: www.medicalschemes.com.

- Table 11: Registration Type Indicators

01	Council for Medical Schemes (CMS)
02	Private Healthcare Industry Standards Committee (PHISC)

- PHISC List of Medical Schemes exempt from registration with Council for Medical Schemes (CMS):
  - PHISC will be responsible for the list of medical schemes that is not required by the Medical Schemes Act to be registered with the CMS.
  - Each medical scheme must be assigned a unique number.
  - A unique number will ensure that each medical scheme in the healthcare community is assigned a unique number to enable electronic transmissions of Claims.

2.3.4 Member Information

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Principal Member Title	5AN	Member title CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims.	Both	CM	CM	CM	CM
Principal Member Surname	20AN	Member Surname	Both	M	M	M	M
Principal Member Initials	5AN	Member Initials. No full stops to be used.	Both	M	M	M	M
Principal Member ID	20AN	In order for the Medical Schemes to search for the main member. If no ID number available then use first six numbers of date of birth.	Both	M	M	M	M
Principal Member VAT Registration Number	20AN	CM: Member is required to register a VAT number. CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims.	Both	CM	CM	CM	CM
Principal Member number	30AN	Principal Member Number	Both	M	M	M	M
Principal Member Address Line 1	25AN	Building CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims. CM: Information must be available to party generating original message i.e. Claim.	Both	CM	CM	CM	CM
Principal Member Address Line 2	25AN	Street Name and Number CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims. CM: Minimum data required regardless of submission method (paper / electronic)	Both	M	M	M	M
Principal Member Address Line 3	25AN	Suburb CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims. CM: Information must be available to party generating original message i.e. Claim.	Both	CM	CM	CM	CM
Principal Member Address Line 4	25AN	Town CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims. CM: Minimum data required regardless of submission method (paper / electronic)	Both	M	M	M	M

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Principal Member Address Line 5	10AN	Postal Code CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims. CM: Minimum data required regardless of submission method (paper / electronic)	Both	M	M	M	M
Principal Member Primary Contact Telephone Country Code	10AN	International standards, need country code CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims. CM: Information must be available to party generating original message i.e. Claim.	Both	CM	CM	CM	CM
Principal Member Primary Contact Telephone Area Code	10AN	Area Code CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims. CM: Information must be available to party generating original message i.e. Claim.	Both	CM	CM	CM	CM
Principal Member Primary Contact Telephone Number	10AN	Telephone Number CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims. CM: Information must be available to party generating original message i.e. Claim.	Both	CM	CM	CM	CM

Note:

- The fields Principal Member Title, Initials and Surname can be concatenated when printed on a paper Claim.
- The fields Principal Member Address 1, 2, 3, 4 and 5 can be concatenated when printed on a paper Claim.
- The fields Principal Member Primary Contact Telephone Country Code, Principal Member Primary Contact Telephone Area Code and Principal Member Primary Contact Telephone Number can be concatenated when printed on a paper Claim or submitting electronic claims.

- Only one Principal member is allowed per Claim.

- When a message is generated that needs to contain postal address information, the following example illustrates how information could be reflected: [Postnet_Suite_1234_Private_Bag_X1234]

  The information illustrated in this example could be split into the two Address Line fields (Address Line 1 and Address Line 2) as:
  [Address Line 1]: [Postnet_Suite_1234]
  [Address Line 2]: [Private_Bag_X1234]

  Alternatively, to comply with the minimum data requirements: [Address Line 2]: [Postnet_1234_PBag_X1234]

2.3.5 Provider Information

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Treating / Attending Provider Personal PCNS number	18AN	Mandatory to have a treating provider at sub-header as the unique invoices relates to the treating provider and patient interaction. Dental Technician Laboratory Claims must reflect the dental laboratory's information. Where the Billing practice is an individual, it is possible that the billing and treating provider's PCNS numbers are the same but should not be understood as a rule. In the instances where the billing practice is a group, partnership or association, the treating provider's PCNS can be different from the Billing Practice's PCNS number. Note that the last 7 characters are the provider's sequential PCNS registration number **but that ALL 13-digits of the PCNS number MUST be included on all claims.** A PCNS number of 13-digits must be submitted - without any spaces or other punctuation marks and keeping the leading zeroes intact. CM: If the treating / attending provider do NOT have a personal PCNS number, then the Professional Council number must be submitted. This field is Mandatory on all Claims, unless there is no PCNS then the Professional Council number becomes mandatory. PCNS mandatory for COID claim.	Both	CM	CM	CM	CM
Treating / Attending Provider Council Registration No.	16AN	CM: A treating provider may not have a personal PCNS number and then must have a Professional Council number. Leading zeroes must not be dropped.	Both	CM	CM	CM	CM
Treating / Attending Provider Council Type	2AN	CM: When a Professional Council number is quoted on a Claim, it must be indicated which professional council's number has been quoted. Refer to Table 1.	Both	CM	CM	CM	CM
Treating Provider Title	5AN	Treating service provider's title. CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims.	Both	CM	CM	CM	CM
Treating Provider Initials	5AN	Treating service provider's Initials. No full stops to be used. CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims.	Both	CM	CM	CM	CM
Treating Provider Surname	20AN	Treating service provider's Surname. CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims.	Both	CM	CM	CM	CM

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Treating Provider Dispensing Licence Number	10AN	Dispensing Provider Licence Number. Recommended by HPCSA to be on the Claim. CM: Whether treating provider is dispensing medication or not.	Both	CM	NOT Required	NOT Required	NOT Required

Note:

- The fields Treating Provider Title, Initials and Surname can be concatenated when printed on a paper Claim.

- Submission / Processing of Claims from group and grouped practices

  - On a single Claim from a Group Practice (Practice type 050) or in the instance where a Billing Practice consists of more than one treating provider, it may become necessary to indicate that different providers treated the patient on the same or different service dates. When this happens, the Provider Information section needs to be repeated for as many times as there are different treating providers.
  - This rule is applicable to both paper and electronic Claims.
  - Referring provider information to be repeated together with repeated treating provider information.
  - Line item numbers must remain continuous from one sub-header to the next and not restart at 1 to ensure unique line items per claim. This is a requirement for automatic reconciliations of electronic remittance advices.

- Table 1: Council Type Indicators

01	HPCSA	Health Professions Council of South Africa
02	AHPCSA	Allied Health Professions Council of South Africa
03	SACSSP	SA Council for Social Services Professions
04	SADTC	SA Dental Technician Council
05	SANC	SA Nursing Council
06	SAPC	SA Pharmacy Council

- There is likelihood that the use of a personal PCNS number in the industry could become unnecessary in the future.

2.3.6 Claim Number Information

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Transaction Type Indicator and Description	7AN / 20AN	The Transaction Type indicates whether the transaction is a: • Normal Claim / COID Claim • Normal Amended Claim / COID Amended Claim • Normal Credit Note / COID Credit Note Table 10 contains a full list of all available Transaction Type Indicators. This field is used to distinguish between the different template codes indicating which form i.e. Claim form or Credit Note form has been submitted. The template code is represented in alpha-numerical characters and is supplied by the PHISC Technical Sub-Committee's Standardised Claim Form Content Workgroup. Where any patient or claim details are altered due to incorrect information on the first Claim only an Amended Claim needs to be supplied. An Amended Claim must always contain both the Original Claim Number as well as the new Claim Number even if these two numbers are the same. A Credit Note will only be supplied if an entire Claim has been reversed. The Credit Notes will be implemented as part of each Medical Scheme's business process at their own discretion. When generating a message i.e. Claim, it is more user-friendly to print in words what kind of document is being presented. Refer to Table 10 for the available Transaction Type Descriptions. CM: Paper. CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims.	Both	M	M	M	M
Resend Indicator and Description	1AN & 4AN	CM: If a previously generated Claim is regenerated then the re-generated Claim need to indicate this. This Indicator indicates that an exact copy or echo of a previous claim is being presented. Paper: Need to print the word 'Copy' Electronic: Need to use 'Y'.	Both	CM	CM	CM	CM
Billing Practice / Supplier Claim / Invoice / Credit Note Number	35AN	This number will be used for reconciliation purposes A reversal must be linked to the original Claim Number. Unique service provider / patient interaction - unique invoice and cannot be repeated for that PCNS combination with invoice number / credit note number. This number could also be the script number when used for Pharmacy claims.	Both	M	M	M	M

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Notation	80AN	"Values are VAT inclusive and are claimed in South African Rands". M: Notation to be added to paper claims CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims.	Both	CM	CM	CM	CM
Authorisation Number	20AN	CM: Based on Medical Scheme requirements.	Both	CM	CM	CM	CM
Supplier / Billing Practice Original Claim / Invoice Number	35AN	This should reference the original Invoice / Claim / Amended Claim Number. CM: Used on Amended Claims and Credit Notes only.	Both	CM	CM	CM	CM
Claim Number Version	2N	This field is used to indicate when a Claim has been changed by a processing party in the trading channel. The original Claim message must be created as a version 0 and each adjustment will increase the version number by 1. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Original Claim Number Version	2N	Must be used on an Amended Claim and a Credit Note to indicate changes and make reference to the original Claim. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Credit Note Version Number	2N	This field is used to indicate when a Credit Note message has been changed by a processing party in the trading channel. The original Credit Note message must be created as a version 0 and each adjustment will increase the version number by 1. CM: Electronic Credit Note only.	Electronic	CM	CM	CM	CM
Claim Response Version Number	2N	This field is used to indicate when a Response message has been changed by a processing party in the trading channel. The original Response message must be created as a version 0 and each adjustment will increase the version number by 1. CM: Electronic Claim Responses only.	Electronic	CM	CM	CM	CM
Credit Note Response Version Number	2N	This field is used to indicate when a Credit Note Response message has been changed by a processing party in the trading channel. The original Credit Note Response message must be created as a version 0 and each adjustment will increase the version number by 1. CM: Electronic Credit Note Responses only.	Electronic	CM	CM	CM	CM
Transaction Type Bar Code	CM	Must be placed on the top right hand corner of paper Claims, repeated on each page. Template Codes supplied by PHISC Standardised Claim Form Forum. CM: Can be used where technology is available.	Paper	CM	CM	CM	CM

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Electronic Response Required Indicator	1AN	This indicator is a **request** from a Billing Practice to a processing intermediary / party asking that although the Billing Practice has submitted a **paper Claim** for reimbursement**,** they would like an **electronic Response** message returned. Please note that the request might not be honoured by the processing intermediary / party. CM: The nature of the indicator is a request that might not be honoured.	Both	CM	CM	CM	CM

Note:

- Table 10: Transaction Type Indicators and Descriptions

Transaction Type Indicator	Transaction Type Description	Additional Information
PHISC01	Claim	Used for both paper and electronic Claims.
PHISC02	Amended Claim	Used for both paper and electronic Claims.
PHISC03	Credit Note	Used for both paper and electronic Credit Notes.
PHISC04	COID Claim	Used to show that the Claim needs to be submitted to the Compensation Commissioner and not the Medical Scheme for reimbursement. Not used for electronic Claims.
PHISC05	COID Amended Claim	Used to show that the Amended Claim needs to be submitted to the Compensation Commissioner and not the Medical Scheme for reimbursement. Not used for electronic Claims.
PHISC06	COID Credit Note	Used to show that the Credit Note needs to be submitted to the Compensation Commissioner and not the Medical Scheme for reimbursement. Not used for electronic Credit Notes.
PHISC07	Response Message	Used for electronic Response messages.

- Global uniqueness of a Message:
    - Three (3) combined pieces of information needs to be considered together when reading and / or interpreting a message. The fields are: (1) Transaction Type Indicator, (2) Billing Practice / Supplier Claim / Invoice / Credit Note Number and (3) Claim Version Number OR Original Claim Version Number OR Credit Note Version Number.
- Amended Claims
    - An Amended Claim is a request to a funder from a provider entity to completely reverse the original Claim (as indicated on the Amended Claim) and then reprocess the Amended Claim message / printout as a new message / printout.
    - It is possible that a funder is unable to find the original Claim message / printout in its database. The funder will in this instance still process the Amended Claim and indicate on their response back that the Credit Note was not done but the Amended Claim was processed.
    - An Amended Claim must be produced and submitted as soon as any information on the previously submitted Claim is changed bearing in mind that it must be for the same Medical Scheme and the Principal member. An Amended Claim implies a Credit Note. This means that the Funder or processor intermediary is required to completely reverse the original Claim, as indicated on the Amended Claim message. The Amended Claim must then be processed as a new Claim. Using this process, the Billing Practice / Supplier does not have to submit a Credit Note to reverse the original Claim and then submit an Amended Claim, only one message will be submitted, namely the Amended Claim.
- Credit Notes
    - A Credit Note is produced when an entire Claim must be reversed for example, same Medical Scheme but for a different Principal Member or different Medical Scheme.
    - In the electronic environment, Credit Notes and Claims / Amended Claims will never be mixed in a batch. A batch will consist of either Claims OR Credit Notes.

- The following activity diagram can be used as an example for generating the Standardised Claim Form (Paper and / or Electronic message(s)).

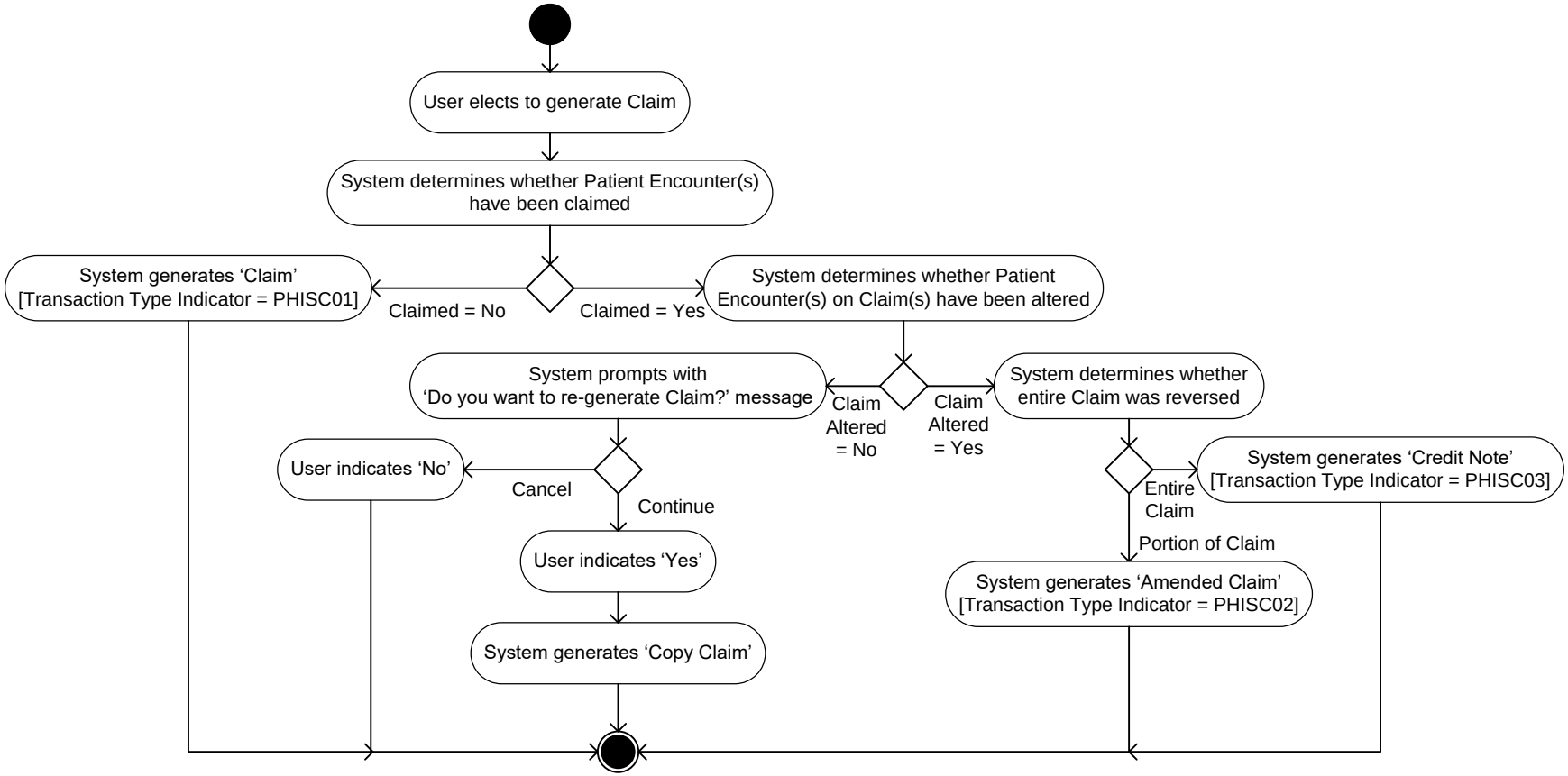

Figure 3: Activity diagram for generating the Standardised Claim Form related documents

2.3.7 Referral and Prescriber Information

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Referring Provider / Prescriber Personal PCNS Number	18AN	CM: Practice Type pathology, radiology, pharmacy and specialists depending on Medical Scheme rules. Mandatory for dental technician laboratory claims in order to indicate the dentist's information. Take note that for each change in the referring service provider, this number must be repeated at a sub-header level. The assumption is that the Prescriber is the Referring provider for pharmacy claims. The personal PCNS number must be present on the Claim and NOT the billing practice's PCNS number. In the instance where a referring provider does not have a personal PCNS number, the provider's professional council number must be included on the Claim.	Both	CM	CM	CM	CM
Referring Provider / Prescriber Council number	16AN	CM: When a service provider does not have a personal PCNS number, the claim must reflect his / her professional council number when a referral is made or when a prescription is issued.	Both	CM	CM	M	CM
Referring Provider / Prescriber Council Type	2AN	CM: When a Professional Council number is quoted on a Claim, it must be indicated which professional council's number has been quoted. Refer to Table 1.	Both	CM	CM	M	CM
Referring Provider / Prescriber Title	5AN	Referring Provider / Prescriber's title. CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims.	Both	CM	CM	CM	CM
Referring Provider / Prescriber Initials	5AN	Referring Provider / Prescriber's Initials. No full stops to be used. CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims.	Both	CM	CM	CM	CM
Referring Provider / Prescriber Surname	20AN	Referring Provider / Prescriber's Surname. CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims.	Both	CM	CM	CM	CM
Diagnosis Code Sequence Number	2N	Use Sequence Number to indicate importance of the diagnosis code. The first ICD-10 code is considered the Primary Diagnosis. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Referring Diagnosis Code(s)	Repeatable 10AN per ICD-10 code	It is mandatory to submit ICD-10 codes linked to procedural codes to be interpreted as the treating / attending service provider's diagnostic codes. It is also mandatory to submit a referring service provider's diagnostic codes, but not at the line level linked to procedural codes. Rather, the referring service provider's details and accompanying diagnoses must be submitted at a higher, summary level. In the case of dental laboratory technicians submitting their own Claims for reimbursement, these practices need to use the same principle by supplying the dentist or dental specialist's details and diagnostic codes as the referral information. Provision must be made for a maximum of 10 ICD-10 codes which means this field is repeated for the number of times there is a diagnosis. The delimiter for electronic Claims will be determined by the electronic standard used i.e. EDIFACT and XML and might differ depending on the format used. The delimiter for paper Claims is a space + forward slash + space between ICD-10 codes. For example A99.99_/_A99.99 (where the underscore represents a space). No indication for primary or secondary diagnosis is required. No dagger or asterisk symbols are required for electronic Claims. A decision is pending from the WHO regarding paper Claims.	Both	M	M	M	M
Referral Reference Number	35AN	The referring provider issues the treating provider with a Referral Reference Number. The purpose of this Number is to indicate to any processing party that the referring and treating providers' information is explicitly linked to the Clinical Encounter that is being presented on a Claim generated and submitted by the treating provider's Billing Practice. The Referral Reference Number must be unique per patient interaction and referral, within the referring provider's Billing Practice. Condition: When there is an arrangement between the referring provider's Billing Practice, treating provider's Billing Practice and the processing party which calls for the submission of the number.	Both	CM	CM	CM	CM

Note:

- The fields Referring Provider / Prescriber Title, Initials and Surname can be concatenated when printed on a paper Claim.

- Submission / Processing of Claims from group and grouped practices

  - On a single Claim from a Group Practice (Practice type 050) or in the instance where a Billing Practice consists of more than one treating provider, it may become necessary to indicate that different referring providers referred the patient for the same or different service dates. When this happens, the Referral Information section needs to be repeated for as many times as there are different treating providers with different referral providers.

  - This rule is applicable to both paper and electronic Claims.

  - Line item numbers must remain continuous from one sub-header to the next and not restart at 1 to ensure unique line items per claim. This is a requirement for automatic reconciliations of electronic remittance advices.

- Table 1: Council Type Indicators

01	HPCSA	Health Professions Council of South Africa
02	AHPCSA	Allied Health Professions Council of South Africa
03	SACSSP	SA Council for Social Services Professions
04	SADTC	SA Dental Technician Council
05	SANC	SA Nursing Council
06	SAPC	SA Pharmacy Council

2.3.8 Patient Information

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Patient Surname	20AN	CM: Mandatory field requirements differ between Medical Schemes. If the data element is mandatory, then the information must be displayed on the member's card.	Both	CM	CM	CM	CM
Patient First Name	20AN	For service providers that do not actually see patients i.e. pathologists, these Billing Practices' Claims must contain either a patient's first name or initials. CM: Mandatory field requirements differ between Medical Schemes. If the data element is mandatory, then the information must be displayed on the member's card. Mandatory for COID claim.	Both	CM	CM	CM	CM
Patient Initials	5AN	For service providers that do not actually see patients i.e. pathologists, these Billing Practices' Claims must contain either a patient's first name or initials. No full stops to be used. CM: Mandatory field requirements differ between Medical Schemes. If the data element is mandatory, then the information must be displayed on the member's card.	Both	CM	CM	CM	CM
Patient ID No.	20AN	For service providers that do not actually see patients i.e. pathologists, these Billing Practices' Claims must contain either a patient's first name or initials. If no ID number available then use first six numbers of date of birth. CM: Mandatory field requirements differ between Medical Schemes. If the data element is mandatory, then the information must be displayed on the member's card. Mandatory for COID claim.	Both	CM	CM	CM	CM
Patient Dependent Code	2AN	CM: Mandatory field requirements differ between Medical Schemes. If the data element is mandatory, then the information must be displayed on the member's card. Leave field blank if not supplied by Fund.	Both	CM	CM	CM	CM
Patient Date of Birth	8N CCYYMMDD	CM: Mandatory field requirements differ between Medical Schemes. If the data element is mandatory, then the information must be displayed on the member's card.	Both	CM	CM	CM	CM
Patient Gender	1AN	Indicate the patient's gender. Refer to Table 20. CM: Mandatory field requirements differ between Medical Schemes. If the data element is mandatory, then the information must be displayed on the member's card.	Both	CM	CM	CM	CM

Note:

- The fields Patient Surname, First Name and Initials can be concatenated when printed on a paper Claim.
- Table 20: Patient Gender Indicators

F	Female
M	Male
U	Unknown

2.3.9 COID Information

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Employer Name	20AN	CM: Required if COID claim. A COID claim must be submitted to the Compensation Commissioner and not the Medical Scheme for reimbursement. COID Claims have not been included in the first Electronic Claim implementation iteration.	Paper	CM	CM	CM	CM
Employer Registration Number	20AN	CM: Required if COID claim. A COID claim must be submitted to the Compensation Commissioner and not the Medical Scheme for reimbursement. COID Claims have not been included in the first Electronic Claim implementation iteration.	Paper	CM	CM	CM	CM
CC Claim Number	20AN	CM: Required if COID claim. A COID claim must be submitted to the Compensation Commissioner and not the Medical Scheme for reimbursement. Employer's registration number could be used as an alternative. COID Claims have not been included in the first Electronic Claim implementation iteration.	Paper	CM	CM	CM	CM
Date of Accident	8N CCYYMMDD	CM: Required if COID claim. A COID claim must be submitted to the Compensation Commissioner and not the Medical Scheme for reimbursement. This is an additional date to the service date. COID Claims have not been included in the first Electronic Claim implementation iteration.	Paper	CM	CM	CM	CM

Note:

- The following information must NOT be reflected on a COID claim:
    - Details of employee's medical scheme
    - PCNS number of referring service provider
- The amounts published by the Compensation Commissioner are calculated exclusive of VAT. The claim for services rendered will be assessed and calculated without VAT. If VAT is applicable and a VAT registration number was quoted, the VAT will be calculated and added to the payment without being rounded off.
- COID monetary values must be rounded off to the nearest 10 cents.

2.3.10 Claim / Billing Event Financial Information

A Claim or Billing Event has been defined as a single Claim for a unique Billing Practice / Supplier for a unique medical scheme member.

In the following table reference is made to three (3) types of Amounts. These different types of Amounts can be described as:-

- Submitted Amounts: Generated by a Supplier / Billing Practice.
- Calculated Amounts: Generated by a Trading Party based on a Funder's Tariff Amount.
- Approved Amounts: Generated by a Payor based on scheme rules of the patient's chosen medical scheme plan.

Please take note that all / any financial fields could contain a negative value and it is the processing party's responsibility to correctly interpret the negative and / or positive values. Refer to Appendix B for examples of how the financial values described in this section are applied.

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Submitted Total Claimed Amount	9N RRRRRRR.CC	This is the amount that the Provider wishes to claim directly from the Payor based on the following calculation: Submitted Claim Amount = Submitted Gross Amount • Submitted Total Discount • Total Collections	Both	M	M	M	M
Calculated Total Claimed Amount		This is the amount which the Processing Intermediary determines that the Billing Practice should be attempting to claim from the Payor at the Billing Event level (i.e. the whole claim) based on the following function: Calculated Claim Amount = Calculated Gross Amount LESS Calculated Total Discount LESS Calculated Total Patient Liable Deductions This element is mandatory if the Processing Intermediary provides Calculated Gross Amount, Calculated Discount and Calculated Total	Electronic	CM	CM	CM	CM
Approved Total Claimed Amount		The amount that the Processing Intermediary determines that the Payor should pay the Billing Practice for the whole Billing Event; i.e. the claim as a whole. Depending on the business rule it can either be a function of the Submitted and Calculated Gross Amounts (e.g. lesser or greater of the two) or the sum of all items' Approved Gross Amounts within the claim. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Submitted Total Gross Amount	9N RRRRRRR.CC	This is the sum of all Clinical Encounter (item level) Submitted Gross Amounts.	Both	M	M	M	M
Calculated Total Gross Amount		This is the sum of all Clinical Encounter (item level) Calculated Gross Amounts. CM: The field is mandatory if it used required to determine the Approved Total Patient Liable Deductions for Electronic Claims.	Electronic	CM	CM	CM	CM
Approved Total Gross Amount		The Processing Intermediary determines this amount as a function of the Submitted and/or Calculated Gross Amount based on the Payor's business rules. To illustrate the options, the Payor may require that the Approved Gross Amount be the lesser or greater of Submitted and Calculated Gross Amount, or a payment arrangement may require the Calculated Gross Amount always be used. CM: Electronic Claims only	Electronic	CM	CM	CM	CM
Submitted Total Discount Amount	9N RRRRRRR.CC	The sum of Submitted Billing Event Discount (at claim level) and Submitted Clinical Encounter Discounts (sum of discounts at item level).	Both	M	M	M	M
Calculated Total Discount Amount		The sum of Calculated Billing Event Discount (at claim level) and Calculated Clinical Encounter Discounts (sum of discounts at item level). CM: This element is mandatory if it is required to determine the Approved Total Discount for Electronic Claims.	Electronic	CM	CM	CM	CM
Approved Total Discount Amount		The sum of Approved Billing Event Discount (at claim level) and Approved Clinical Encounter Discounts (sum of discounts at item level). This is the total discount used to calculate the Approved Claim Amount for the whole claim. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Submitted Total VAT Amount	9N RRRRRRR.CC	The VAT amount should be calculated on the Submitted Total Claimed Amount field's value. M: Paper Claims CM: Field become mandatory in the XSD because the XML could be converted to produce a paper Claim at any stage in the trading party channel for Electronic Claims.	Both	CM	CM	CM	CM
Calculated Total VAT Amount		The VAT amount should be calculated on the Calculated Total Claimed Amount field's value. CM: Electronic claims only.	Electronic	CM	CM	CM	CM
Approved Total VAT Amount		The VAT amount should be calculated on the Approved Total Claimed Amount field's value. CM: Electronic claims only.	Electronic	CM	CM	CM	CM
Submitted Payment Indicator	1AN	Pay member = "N" – provider will get paid Pay member = "Y" – member will get paid This is not a receipt but indicates the preference on whether the practice or the member must be paid by the Medical Scheme. CM: This field is only used to denote FULL payments and not partial payments.	Both	CM	CM	CM	CM

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Submitted Total Collection Amount	9N RRRRRRR.CC	The sum of Submitted Billing Event Collections (at claim level) and Submitted Clinical Encounter Collections (sum of collections at item level).	Both	M	M	M	M
Calculated Total Deduction Amount	9N RRRRRRR.CC	This is the sum of all deductions which the Processing Intermediary deems the Billing Practice should have collected from the patient at the point-of-care. CM: The field is mandatory if it used required to determine the Approved Gross Amount for Electronic Claims.	Electronic	CM	CM	CM	CM
Approved Total Deduction Amount		The Processing Intermediary determines this amount as a function of the Submitted Total Collections and/or Calculated Total Patient Liable Deductions based on the Payor's business rules. To illustrate the usage, this could either be : • Equal to the Submitted Total Collections (the Processing Intermediary accepts the upfront collection and will settle differences directly with the member) • Equal to the Calculated Total Patient Liable Deduction (the Processing Intermediary enforces the calculated amount and expects the Billing Practice to settle differences with the member), or In the case of a patient Claim (DoPayMember = True), equal to the difference of Approved Gross Amount less Approved Total Discount; i.e. the Processing Intermediaries' view of an approved patient claim). CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Due By Payor To Member	9N RRRRRRR.CC	This is the amount which the Processing Intermediary determines is due by Payor to Member to settle any excessive out-of-pocket expenses (due to excessive collections by the provider). A negative number in this element means the Billing Practice under-collected from the member, the Payor will reimburse the Billing Practice on the member's behalf and that the member is due the indicated amount to the Payor. See note A below. Due By Payor To Member = Approved Total Patient Liable Deductions – Calculated Total Patient Liable Deductions CM: Electronic Claims only	Electronic	CM	CM	CM	CM
Due By Practice To Member	9N RRRRRRR.CC	This is the amount which the Processing Intermediary determines is due by the Billing Practice to the Member to settle any excessive out-of-pocket expenses (due to excessive collections by the Billing Practice). A negative value in this element means the Billing Practice has under-collected and the member still owes the Billing Practice an outstanding amount as determined by the Processing Intermediary. See note A below. Due By Billing Practice To Member = Total Collections – Approved Total Patient Liable Deductions. CM: Electronic Claims only	Electronic	CM	CM	CM	CM

2.3.11 Transaction Event Financial Information

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Submitted Transaction Event Discount Amount	9N RRRRRRR.CC	The billing practice uses this element to specify the discount which was given to the patient at the point-of-care for the claim level as a whole i.e. not at the per item level. If no discount was given, then this element must contain zero Rands.	Both	M	M	M	M
Calculated Transaction Event Discount Amount		The processing intermediary indicates the discount which the billing practice should have extended at the point-of-care on the **claim level as a whole** i.e. not at the per item level, and is based on defined business rules or contractual agreements on the transaction event level. CM: This element is mandatory if it is required to determine the Approved Transaction Event Discount for electronic Claims.	Electronic	CM	CM	CM	CM
Approved Transaction Event Discount Amount		The processing intermediary determines this amount as a function of the Submitted and/or Calculated Transaction Event Discount based on the payor's business rules. To illustrate, it could be the lesser or greater of the Submitted or Calculated Transaction Event Discount. CM: Electronic Claims only	Electronic	CM	CM	CM	CM
Deduction Type Indicator	2N	Refer to Table 14. CM: This is an attribute of Deductions and is mandatory if Deductions are included in the message on Electronic Claims. Deduction Type is enumerated as follows: • Levy Also known as a deductible, the levy is a fee determined by the Payor which is due by the member/patient to the Billing Practice at the point-of-care. As a demand-side control of expenditure through an out-of-pocket expense levied on the member/patient, it is typically a fixed Rand or percentage value calculated at claim or item level according to the Payor's registered rules. The levy amount is typically not refunded from the Payor's risk benefit but might be reimbursed from the member's savings benefit. • Co-payment Similar to a Levy, however it is a variable fee as a function of factors that the Processing Intermediary is privy to; such as patient benefits available, rules unique to a particular patient / service / product combination. For example, in the case where the Billing Practice can legitimately claim R65 from the Payor, but where the patient runs out of benefit or a limit applies to the extent that there is a R5 shortfall; then the Co-Payment would be R5.	Electronic	CM	CM	CM	CM
Submitted Transaction Event Collection Amount	9N RRRRRRR.CC	CM: The Billing Practice uses this element to specify the collections taken or intended to be taken from the patient at the point-of-care for the **claim level or transaction event** i.e. not at the line item level. If no collection was made or intended to be made, then this element will indicate zero Rands.	Both	CM	CM	CM	CM

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Calculated Transaction Event Deduction Amount	9N RRRRRRR.CC	CM: As a repeatable element, the Processing Intermediary uses it to explain the breakdown of the Calculated Total Patient Liable Deductions and is therefore only mandatory if required by the Processing Intermediary or Payor for Electronic Claims. Refer to Deduction Type Indicator.	Electronic	CM	CM	CM	CM
Sum Submitted Clinical Encounter Discount Amounts	9N RRRRRRR.CC	The sum of all item level Submitted Discounts CM: Electronic Claims only Electronic	Electronic	CM	CM	CM	CM
Sum Calculated Clinical Encounter Discount Amounts		The sum of all item level Calculated Discounts. CM: Electronic Claims only Electronic CM: This element is mandatory if it is required to determine the Approved Clinical Encounter Discounts for Electronic Claims.	Electronic	CM	CM	CM	CM
Sum Approved Clinical Encounter Discount Amounts		The Processing Intermediary determines this amount as a function of the Submitted and/or Calculated Clinical Encounter Discounts or merely the sum of item level Approved Discounts based on the Payor's business rules. To illustrate the options, it could be the lesser or greater of the Submitted or Calculated Clinical Encounter Discounts or merely the sum of the item level Approved Discounts which in themselves could be the lesser or greater of the item level Submitted or Calculated Discounts. CM: Electronic Claims only Electronic	Electronic	CM	CM	CM	CM
Sum Submitted Clinical Encounter Collection Amounts	9N RRRRRRR.CC	The sum of all item level Submitted Collections. CM: Electronic Claims only Electronic	Electronic	CM	CM	CM	CM
Sum Calculated Clinical Encounter Deduction Amounts	9N RRRRRRR.CC	The sum of all item level Calculated Total Deductions. CM: This element is mandatory if item level Calculated Total Deductions exist for Electronic Claims.	Electronic	CM	CM	CM	CM

Note:

- Currency symbols to be omitted on both paper and electronic claims.
- All values quoted will be VAT inclusive.
- Refer to the information documented under section 2.6 Transaction Event: Claim Number Information: Notation field regarding the message “Values are VAT inclusive and claimed in South African Rands” that needs to print on a paper Claim.
- Table 14: Deduction Type Indicators

01	Levy
02	Co-payment

2.3.12 Treatment Information

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Date of Service, Treatment Start Date and Time	8N CCYYMMDD 5AN HH:MM	Service Date is mandatory at item level. Default the value to the date of service. Service Time is part of the same field but is optional. If not captured, then it should default to zeroes. Use 24-hour clock to indicate time. Mandatory for COID claim. Treatment start date and time is mandatory where time modifiers are used by surgeons and anaesthetists.	Both	M	M	M	M
Treatment End Date and Time	8N CCYYMMDD 5AN HH:MM	Could be used by surgeons, anaesthesiologists, anaesthetists, physiotherapists or where time is required for tariffs or modifiers. Default the value to the date of service. Service Time is part of the same field but is optional. If not captured, then it should default to zeroes. Use 24-hour clock to indicate time.	Both	M	M	M	M
Patient Place of Service Code	3N	Mandatory for NHRPL, CPT and NAPPI codes. Use Southern Africa Place of Service Codes refers to Table 4.	Both	M	M	M	M
Item/Tariff Authorisation No.	20AN	CM: Dependent on the Medical Scheme requirements	Both	CM	CM	CM	CM
Billing Practice Line Item Identifier	3N	Best practice: Sequential numbers incrementing with one. This is the original sequential line number generated by the billing practice for the particular claim and must not be changed by any other party. This field does not have to be padded with zeroes e.g. 001 vs. 1 for electronic Claims, but it is advised for paper Claims.	Both	M	M	M	M
Trading Party Line Item Identifier	3N	Best practice: Sequential numbers. This is the original sequential line number generated by any party in the transacting channel where the Billing Practice's Original Claim has been altered through adding line items and / or re-sequencing line items. The Billing Practice's PMA must also populate this field upon creating the Claim using the same number as in the Billing Practice Line Item Identifier data element. If no modifications are made to the original Claim, then the Trading Party should leave this field unchanged. This field does not have to be padded with zeroes e.g. 001 vs. 1 for electronic Claims but it is advised for paper Claims. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
System Reference Number	32AN	This is a system generated number to uniquely identify lines. If this field is populated, it must be echoed back in any / all electronic response messages. CM: Ability of system to generate information for Electronic Claims only.	Electronic	CM	CM	CM	CM

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Procedural Coding Standard Indicator	3N	This relates to the billing coding structure used where procedural codes are billed. Each line item on a claim MUST have a Procedural Coding Standard Indicator with no exceptions for any practice type or claim type. The correct use of this Indicator will be agreed upon by the participating parties. • Option 1 would be that '01 NHRPL' would be used for all appropriate procedural codes and '07 SAMA' would be used only where there are no NHRPL equivalent procedural code. • Option 2 would be to use '07 SAMA' for all line items. Refer to Table 5.	Both	M	M	M	M
Item Type Indicator	2AN	This indicator identifies the code composition per line item. Refer to Table 6.	Both	M	M	M	M
Procedure Code	10AN	This is the actual code as identified in the Item Type Indicator field. Mandatory unless the item type is set to type 01 Product Code only or 05 Modifier Code only. If this is the case then this field would be blank and the relevant fields would be populated. Please note type 01 Product Code only is dependent on the dispensing tariffs and therefore for dispensing doctors there would always be a tariff associated with a Product Code i.e. NAPPI.	Both	M	M	M	M
Modifier Code	6AN	Dependent on the Modifier Submission Standard and whether the tariff quantity can be used. Five modifiers per line item. Please refer to section 2.4 Modifier Submission Standardisation for detailed information on Modifier submission guidelines. CM: Practice Type will determine whether the service provider will use modifiers on codes or not.	Both	CM	NOT Required	CM	CM
Product Coding Standard Indicator	3N	This relates to the product coding structure used where material / medication items are billed. Refer to Table 15. CM: Mandatory when a product code is specified.	Both	CM	CM	CM	CM
Product Code	15AN	Product code becomes mandatory depending on the tariff used and whether medication has been dispensed. CM: When products are used during treatment or when products are dispensed to patients, these item lines on the Claim need to reflect Product codes.	Both	CM	CM	CM	CM
Description / Narration of item	50AN	Description of the item CM: Description / Narration of item not required when claiming NAPPI items. Not required when submitting either an Information or Reduction modifier.	Both	CM	CM	CM	CM

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Quantity	8N	The unit type used to measure an item is defined by the procedural code, modifier and / or NAPPI code. A procedure's Clinical Units / Relative Value Units (RVUs) must never be quoted on claims. For example: ○ In the case of a modifier code i.e. GP anaesthetic modifier 0023, the Quantity field will indicate the time measured in minutes. ○ In the case of a NAPPI item the Quantity field will indicate the number of tablets prescribed / dispensed. ○ In the case of a NAPPI item the Quantity field will indicate the number of vials used to give a patient an injection. Paper and Electronic claims to only use a maximum of 2 decimal places where and if required For example: 2 OR 2.5 OR 2.55 A decimal full stop is not implied and MUST be indicated when required. The description for the procedural code or modifier will determine the unit type to be used on a Claim.	Both	M	M	M	M
Patient Height	3N	In order to calculate the patient's Body Mass Index (BMI). Height is specified in centimetres with no decimals and rounded down. CM: Required when quoting Medical Practitioner modifier 0018 on a claim.	Both	CM	Not required	Not required	Not required
Patient Weight	6N	In order to calculate the patient's Body Mass Index (BMI). Weight is specified in grams with no decimals and rounded down. CM: Required when quoting Medical Practitioner modifier 0018 or 0019 on a claim.	Both	CM	Not required	Not required	Not required
Benefit Type Indicator	3AN	CM: Medical Fund and / or 3rd party specific. PHISC maintains the list of Benefit Type Indicator. This data element Indicates from which patient benefits the claim and / or line item should be reimbursed from. On Claim level, only a single Benefit Type Indicator needs to be populated. However, on Line item level, a further distinction needs to be made between Submitted, Calculated and Approved Benefit Type Indicators. Refer to Table 13.	Both	CM	CM	CM	CM

Note:

- Submission / Processing of Claims from group and grouped practices
  - On a single Claim from a Group Practice (Practice type 050) or in the instance where a Billing Practice consists of more than one treating provider, it may become necessary to indicate that different referring providers referred the patient for the same or different service dates. When this happens, the Referral Information section needs to be repeated for as many times as there are different treating providers with different referral providers.
  - This rule is applicable to both paper and electronic Claims.
  - Line item numbers must remain continuous from one sub-header to the next and not restart at 1 to ensure unique line items per claim. This is a requirement for automatic reconciliations of electronic remittance advices.

- Table 4: Place of Service Codes Indicators

21	Inpatient Hospital	52	Acute psychiatric facility
51	Inpatient psychiatric hospital	53	Out of hospital Mental Health Clinic
61	Inpatient rehabilitation facility	55	Substance Rehabilitation
11	Consulting room	56	Halfway House
12	Home	62	Outpatient rehabilitation
22	Hospital Outpatient facility	65	Dialysis Centre
23	Casualty	66	Radiotherapy Treatment Centre
24	Day Clinic / Hospital	68	Chemotherapy Treatment Centre
25	Birthing Centre	70	Oncology Centre
26	Military treatment facility	71	State or Local Public Health Clinic
31	Step-down	72	Rural Health Clinic
32	Nursing Home	81	Private Laboratory
33	Chronic Psychiatric Facility	84	Independent Pharmacy
34	Hospice	85	Hospital Pharmacy
41	Ambulance – Land	99	Other unlisted facility
42	Ambulance – Air or Water		

- Table 5: Procedural Coding Standard Indicators

01	NHRPL
02	CPT / CCSA
03	CDT
04	Proprietary Code Set
05	SAOA
06	SAOPA
07	SAMA (South African Medical Association)

- Table 15: Product Coding Standard Indicators

01	NAPPI
02	SAOA
03	SAOPA
04	Proprietary Code Set

- Table 6: Item Type Indicators

01	Product Code only
02	Tariff / Procedure Code only
03	Tariff / Procedure Code and Modifier Code
04	Tariff / Procedure Code and Product Code
05	Modifier Code only
06	Dental Tariff / Procedure Code and Dental Laboratory Tariff / Procedure Code

Item Type Indicators 03, 05 and 06 will never be used on Optometry Claims.

- Table 13: Benefit Type Indicators

001	Acute	011	Life Sustaining
002	Appliances	012	Life Threatening
003	Chemotherapy	013	Major Medical
004	Chronic	014	Oncology
005	Dialysis	015	Organ Transplant
006	Ex Gratia	016	Over-The-Counter (OTC)
007	General Practitioners	017	Prosthesis
008	HIV	018	Prescribed Minimum Benefits (PMB)
009	Hospital	019	Specialist
010	Life Compromising	020	Trauma

2.3.13 Additional Information for Dental Items

Principle followed for Dental Claims:

- Multiple procedural codes per Claim.
- Multiple tooth numbers (when required) per procedural code.
- Multiple tooth surfaces per tooth number.
- Multiple Dental Laboratory work per claim.

2.3.13.1 Additional Information for Dental Procedural items

The following information will only be required when a dental procedural code is performed and a Claim is submitted.

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Tooth Number	3AN	CM: Dental Claim specific where procedural codes require tooth number specification. A maximum of eight tooth numbers must be catered for per item line and will never be required on an 8099 item. The dental procedural code indicates when specifying tooth numbers are required. Super-numerary tooth numbers are indicated with 2 numeric tooth number followed by an “S”, e.g. 23S Paper Claims: Concatenate the string of tooth numbers using no separators. Electronic Claims: No concatenation required for XML generated claim. Refer to XML document for more information.	Both	CM	NOT Required	NOT Required	NOT Required
Tooth Surface	5AN	CM: Dental Claim specific where Medical Scheme requires tooth surface specification for specific procedural codes. Surface Number must concatenate with Tooth No. field. Will submit Tooth number + Surface – Repeatable and will never be required on an 8099 item. Refer to Table 12	Both	CM	NOT Required	NOT Required	NOT Required

Note:

- Tooth identification and designation of areas of the oral cavity is compulsory for all Claims rendered for reimbursement.
- Tooth identification is applicable to procedure codes identified with the letter (T) in the NHRPL.
- The International Standards Organisation (ISO) in collaboration with the FDI designated system for teeth and areas of the oral cavity should be used.

- FDI tooth numbering chart:

			55	54	53	52	51	61	62	63	64	65				Deciduous (milk) teeth
18	17	16	15	14	13	12	11	21	22	23	24	25	26	27	28	Permanent teeth
48	47	46	45	44	43	42	41	31	32	33	34	35	36	37	38	Permanent teeth
			85	84	83	82	81	71	72	73	74	75				Deciduous (milk) teeth

- Table 12: Abbreviations for Tooth Surfaces

B	Buccal (Vestibular)
D	Distal
L	Labial (Vestibular)
L	Lingual
M	Mesial
O	Occlusal (or incisal)
P	Palatal

Note:
The tooth surfaces that can be used together are dependent on the position of the tooth that it is describing. Therefore having two abbreviations that are the same (L = Labial and L = Lingual) should not pose any problems when represented on a Claim.

2.3.13.2 Additional Information for Dental Laboratory items

The following information will only be required when a dental laboratory code (8099) is submitted by a dentist or dental specialist. As a best practice recommendation, the dental practitioner should always claim a dental laboratory code (8099) accompanied with the relevant dental technician procedural code(s). In some instances i.e. when a patient elects not to complete treatment, the dental laboratory code (8099) could be submitted on its own.

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Dental Laboratory Procedure Code	10AN	CM: Dental Claim specific for dental laboratory. This is the actual dental laboratory item's procedure code as identified in the Item Type Indicator field. Mandatory when submitting dental procedure code 8099.	Both	CM	NOT Required	NOT Required	NOT Required
Dental Laboratory Technician's Personal PCNS number	18AN	CM: Dental Claim specific for dental laboratory. Mandatory for 8099. Only one PCNS number per 8099.	Both	CM	NOT Required	NOT Required	NOT Required
Dental Laboratory Technician's Council Registration Number	16AN	CM: Dental Claim specific for dental laboratory. Mandatory for 8099. A Dental Technician Laboratory may not have a PCNS number and then must have a Professional Council number. Leading zeroes must not be dropped. Only one SA Dental Technician Council (SADTC) registration number per 8099 submission.	Both	CM	NOT Required	NOT Required	NOT Required
Dental Laboratory Technician Council Type	2AN	CM: Dental Claim specific for dental laboratory. When a Professional Council number is quoted on a Claim, it must be indicated which professional council's number has been quoted. Refer to Table 1.	Both	CM	NOT Required	NOT Required	NOT Required
Dental Laboratory Reference Number	32AN	CM: Dental Claim specific for dental laboratory. This is the reference number that the Dental Laboratory will use to uniquely identify an 8099 laboratory dental laboratory request (original invoice of the Dental Technician). This number will be used for reconciliation purposes between the dental practitioner and the dental technician laboratory. Uniquely identify dental technician / patient interaction. This unique Dental Laboratory Reference number cannot be repeated for the specific PCNS / council registration number combination. This field becomes crucial when medical schemes and / or intermediaries check for duplicate Line items. Certain processing parties may choose to store this information for auditing purposes.	Both	CM	NOT Required	NOT Required	NOT Required

Note:

- Table 1: Council Type Indicators

01	HPCSA	Health Professions Council of South Africa
02	AHPCSA	Allied Health Professions Council of South Africa
03	SACSSP	SA Council for Social Services Professions
04	SADTC	SA Dental Technician Council
05	SANC	SA Nursing Council
06	SAPC	SA Pharmacy Council

2.3.14 Additional Information for Pathology Items

Principle followed for Pathology Claims:

- Multiple pathology laboratory reference numbers per Claim, with at least one (1) pathology reference number per Claim.

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Pathology Laboratory Requisition / Accession Number	32AN	CM: Pathology Claim specific This field denotes that a requisition / accession have been received for pathology laboratory work. No pathology work can be done without a requisition / accession number. A Claim can contain multiple pathology laboratory requisition / accession numbers that allows for the billing of multiple tests without them being necessarily viewed as duplicates by medical schemes and / or processing intermediaries. The only way to detect a true erroneous duplicate claim is for the processing party to consider the submitted procedural code + procedural code's description + billing practice's PCNS number. The principles of (1) one requisition / accession number per line item and (2) possible repetition of a single requisition / accession number within a single billing event will be applied. This number will be used for reconciliation purposes. This number uniquely identifies a pathology laboratory / patient interaction.	Both	NOT Required	NOT Required	NOT Required	CM

Note:

- SNOMED / LOINC codes are not required on pathology Claims because it is clinical information not required for reimbursement purposes.

2.3.15 Additional Information for Dispensing Items

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Daily Dosage	6N	CM: Claim contains Dispensed medication Mandatory when there are ethical items being dispensed.	Both	CM	CM	CM	NOT Required
Days of Therapy	3N	CM: Claim contains Dispensed medication Mandatory when there are ethical items being dispensed.	Both	CM	CM	CM	NOT Required
Basis of DOT	2N	CM: Claim contains Dispensed medication Mandatory when there are ethicals being dispensed. Refer to Table 7.	Both	CM	CM	CM	NOT Required
Mixture Group Identifier	2AN	CM: Claim contains Dispensed medication that includes a mixture. Repeated for all items within the mixture group. Unique number per mixture must be used.	Both	CM	CM	NOT Required	NOT Required
Mixture Code	2N	CM: Claim contains Dispensed medication that includes a mixture. Mandatory when a mixture is being prepared and dispensed Refer to Table 8.	Both	CM	CM	NOT Required	NOT Required
Repeat Number	2N	CM: Claim contains Dispensed medication. The repeat is in the sequence of repeats. A repeat number for that specific script.	Both	CM	CM	CM	NOT Required
Number of Repeats Authorised	2N	CM: Claim contains Dispensed medication The number of repeats authorised by the service provider.	Both	CM	CM	CM	NOT Required

Note:

- Table 7: Basis of Days of Therapy Indicators

00	Not Specified
01	Implicit Usage
02	Dispensers Estimate
03	Doctors Directions

- Table 8: Mixture Code Indicators

00	Not an ingredient of a mixture
01	Item is an ingredient of a mixture
02	Item is an ingredient of a mixture and includes mixture fee

Please take note that sequence of mixture items are not indicated by the information represented in Table 8 but rather where the mixture fee has been applied.

2.3.16 Item Diagnosis Information

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Diagnosis Code	Repeatable 10AN per ICD-10 code	It is mandatory to submit ICD-10 codes linked to procedural codes to be interpreted as the treating / attending service provider's diagnostic codes. It is also mandatory to submit a referring service provider's diagnostic codes, but not at the line level linked to procedural codes. Rather, the referring service provider's details and accompanying diagnoses must be submitted at a higher, summary level. Provision must be made for a maximum of 10 ICD-10 codes which means this field is repeated for the number of times there is a diagnosis. The delimiter for electronic Claims will be determined by the electronic standard used i.e. EDIFACT and XML and might differ depending on the format used. The delimiter for paper Claims is a space + forward slash + space between ICD-10 codes. For example A99.99_/_A99.99 (where the underscore represents a space). No indication for primary or secondary diagnosis is required. No dagger or asterisk symbols are required for electronic Claims. A decision is pending from the WHO regarding paper Claims.	Both	M	M	M	M
Diagnosis Code Sequence Number	2N	Use Sequence Number to indicate importance of the diagnosis code and to preserve the capture sequence. The first ICD-10 code is considered the Primary Diagnosis. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM

2.3.17 Item Assistant Information

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Assistant Personal PCNS Number	18AN	CM: Claim contains Assistant information Repeatable field to cater for multiple assistants per line, with no limit. The personal PCNS number must be present on the Claim and NOT the billing practice's PCNS number. In the instance where an assistant do not have a personal PCNS number, the assistant's professional council number must be included on the Claim.	Both	CM	NOT Required	NOT Required	NOT Required
Assistant Council Registration No.	16AN	CM: Claim contains Assistant information An assistant may not have a personal PCNS number and then must have a Professional Council number. Leading zeroes must not be dropped. . Repeatable field to cater for multiple assistants per line, with no limit.	Both	CM	NOT Required	NOT Required	NOT Required
Assistant Council Type	2AN	CM: Claim contains Assistant information When a Professional Council number is quoted on a claim, it must be indicated which professional council's number has been quoted. Refer to Table 1.	Both	CM	NOT Required	NOT Required	NOT Required

Note:

- Table 1: Council Type Indicators

01	HPCSA	Health Professions Council of South Africa
02	AHPCSA	Allied Health Professions Council of South Africa
03	SACSSP	SA Council for Social Services Professions
04	SADTC	SA Dental Technician Council
05	SANC	SA Nursing Council
06	SAPC	SA Pharmacy Council

2.3.18 Item Optometric Information

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Visual Acuity	nn/nn +n OR nn/nn –n	CM: Optometric Claim specific Mandatory for left and right eyes.	Both	NOT Required	NOT Required	CM	NOT Required
Visual Fields	AN (Free text)	CM: Optometric Claim specific Mandatory for left and right eyes. Service Provider to confirm whether any other pattern / encoding could be used.	Both	NOT Required	NOT Required	CM	NOT Required
Intra-Ocular Pressure	5AN (nn.nn)	CM: Optometric Claim specific Mandatory for left and right eyes. Measurement unit is implied to be millimetres.	Both	NOT Required	NOT Required	CM	NOT Required
Laboratory Number	32AN	CM: Optometric Claim and Scheme rule specific. PPN requires information This field is required when lenses are ordered.	Both	NOT Required	NOT Required	CM	NOT Required
Laboratory Order Number	32AN	CM: Optometric Claim and Scheme rule specific. PPN requires information This field is required when lenses are ordered.	Both	NOT Required	NOT Required	CM	NOT Required
Lens Prescription: Eye Indicator	1AN	CM: Optometric Claim specific. This field is required when lenses are ordered. Expected value is R or L	Both	NOT Required	NOT Required	CM	NOT Required
Lens Prescription: Sphere	+nn.nn or -nn.nn	CM: Optometric Claim specific. This field is required when lenses are ordered. Field is required for left and / or right eye.	Both	NOT Required	NOT Required	CM	NOT Required
Lens Prescription: Cylinder	nn.nn	CM: Optometric Claim specific. This field is required when lenses are ordered. Field is required for left and / or right eye. Value is always assumed negative	Both	NOT Required	NOT Required	CM	NOT Required
Lens Prescription: Axis	3N	CM: Optometric Claim specific. This field is required when lenses are ordered and Cylinder is present. Field is required for left and / or right eye. Expected value range is 1 – 180 (degrees)	Both	NOT Required	NOT Required	CM	NOT Required
Lens Prescription: Prism	2N	CM: Optometric Claim specific. This field is required when lenses are ordered. Field is required for left and / or right eye. Expected value range is 1-99	Both	NOT Required	NOT Required	CM	NOT Required
Lens Prescription: Prism Base	2AN	CM: Optometric Claim specific. This field is required when lenses are ordered and Prism is present. Field is required for left and / or right eye. Expected values are IU, ID, OU, OD, I, O, U or D	Both	NOT Required	NOT Required	CM	NOT Required

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Lens Prescription: Reading add	nn.nn	CM: Optometric Claim specific. This field is required when lenses are ordered. Field is required for left and / or right eye. Value always assumed positive	Both	NOT Required	NOT Required	CM	NOT Required
NVS Tag Number	15N	CM: Optometric Claim and Scheme rule specific. This field is required when lenses are ordered. This field is mandatory for Frame procedural codes where each frame will have its own NVS Tag Number.	Both	NOT Required	NOT Required	CM	NOT Required

Note:

- The type of lens e.g. glass, plastic or contacts, used is implied by the procedural code that is used on the Claim.
- Standard to be used on paper Claims:
  - Left eye: Spherical, Cylindrical, Axis, Add, Prism, Base
  - Right eye: Spherical, Cylindrical, Axis, Add, Prism, Base

2.3.19 Line Item Financial Information

In the following table reference is made to three (3) types of Amounts. These different types of Amounts can be described as:-

- Submitted Amounts: Generated by a Supplier / Billing Practice.
- Calculated Amounts: Generated by a Trading Party based on a Funder's Tariff Amount.
- Approved Amounts: Generated by a Payor based on scheme rules of the patient's chosen medical scheme plan.

Please take note that all / any financial fields could contain a negative value and it is the processing party's responsibility to correctly interpret the negative and / or positive values. Refer to Appendix B for examples of how the financial values described in this section are applied.

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Submitted Item Claimed Amount	9N RRRRRRR.CC	This is the amount that the Provider wishes to claim directly from the Payor based on the following calculation: Submitted Claim Amount = Submitted Gross Amount MINUS Submitted Discount MINUS Collections	Both	M	M	M	M
Calculated Item Claimed Amount		This is the amount which the Processing Intermediary determines that the Provider should be attempting to claim from the Payor based on the following function: Calculated Claim Amount = Calculated Gross Amount MINUS Calculated Discount MINUS Calculated Total Patient Liable Deductions CM: This element is mandatory if the Processing Intermediary provides Calculated Gross Amount, Calculated Discount and Calculated Total Patient Liable Deductions for Electronic Claims.	Electronic	CM	CM	CM	CM
Approved Item Claimed Amount		This is the amount that the Processing Intermediary determines that the Payor should pay the Provider for the item. It is calculated as follows: Approved Claim Amount = Approved Gross Amount MINUS Approved Discount MINUS Approved Total Patient Liable Deductions. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Submitted Item Base Cost Amount	9N RRRRRRR.CC	The actual input cost of the item submitted by the Billing Practice. This may be based on tariff guidelines provided by health professions councils, privately determined rates, published tariffs or mutually agreed payment arrangements.	Both	M	M	M	M
Calculated Item Base Cost Amount		This is the base cost which the Processing Intermediary believes is appropriate for the item, a legal requirement or which was agreed with the Billing Practice through contractual arrangement. This may be based on tariff guidelines provided by health professions councils, medical aid rates, published tariffs or mutually agreed payment arrangements. CM: The field is mandatory if the Processing Intermediary intends explaining how the Calculated Gross Amount was determined and/or if it is required in determining the Approved Base Cost for Electronic Claims.	Electronic	CM	CM	CM	CM
Approved Item Base Cost Amount		The Processing Intermediary determines this amount as a function of the Submitted and/or Calculated Base Cost based on the Payor's business rules. CM: The field is mandatory if the Approved Gross Amount is based on it for Electronic Claims.	Electronic	CM	CM	CM	CM
Submitted Item Mark-up Amount	9N RRRRRRR.CC	This element is used by the Billing Practice to specify the mark-up in Rand value applied to the Submitted Base Cost to arrive at the Submitted Gross Amount. CM: The element is mandatory if required by the Processing Intermediary or Payor for electronic Claims.	Both	CM	CM	CM	CM
Calculated Item Mark-up Amount		This element is used by the Processing Intermediary to specify the mark-up in Rand value applied to the Calculated Base Cost to arrive at the Calculated Gross Amount. CM: This element is mandatory if the Processing Intermediary intends providing Calculated Base Cost to explain how the Calculated Gross was determined. The field is also mandatory if it is required to determine the Approved Mark-Up for electronic Claims.	Electronic	CM	CM	CM	CM
Approved Item Mark-up Amount		The Processing Intermediary determines this amount as a function of the Submitted and/or Calculated Mark-Up based on the Payor's business rules. CM: The field is mandatory if the Approved Gross Amount is based on it for electronic Claims.	Electronic	CM	CM	CM	CM

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Submitted Item Professional Fee Amount	9N RRRRRRR.CC	This element is used by the Billing Practice to indicate the Professional Fee for a service and is included in the Submitted Gross Amount. A typical example is the dispensing fee charged by dispensing service providers. CM: The element is mandatory if required by the Processing Intermediary or Payor for Electronic Claims.	Electronic	CM	CM	CM	CM
Calculated Item Professional Fee Amount		This element is used by the Processing Intermediary to specify the Professional Fee which should have been used by the Billing Practice based on legal requirements or contractual arrangements. CM: The field is mandatory if the Processing Intermediary intends explaining how the Calculated Gross Amount was determined and/or if it is required in determining the Approved Professional Fee for electronic Claims.	Electronic	CM	CM	CM	CM
Approved Item Professional Fee Amount		The Processing Intermediary determines this amount as a function of the Submitted and/or Calculated Professional Fee based on the Payor's business rules. CM: The field is mandatory if the Approved Gross Amount is based on it for Electronic Claims.	Electronic	CM	CM	CM	CM
Submitted Item Gross Amount	9N RRRRRRR.CC	The Gross amount deemed chargeable by the Billing Practice based on input costs (Base Cost), mark-up or professional fee. Submitted Gross Amount = Submitted Base Cost PLUS Submitted Mark-Up or Professional Fee	Both	M	M	M	M
Calculated Item Gross Amount		This is the Gross amount which the Processing Intermediary deems should have been charged by the Billing Practice based on Payor business rules, patient plan choices and Billing Practice / Payor contractual arrangements. This is the maximum that the Payor would pay for the item. This element is mandatory if it is required to determine the Approved Gross Amount. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Approved Item Gross Amount		The Processing Intermediary determines this amount as a function of the Submitted and/or Calculated Gross Amount based on the Payor's business rules. To illustrate, the Payor may require that the Approved Gross Amount be the lesser of Submitted and Calculated Gross Amount, or a payment arrangement may require the Calculated Gross Amount always be used, or that Approved Gross Amount be calculated from Approved Base Cost, Approved Mark-up and Approved Professional Fee. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Submitted Item Discount Amount	9N RRRRRRR.CC	The Billing Practice uses this element to specify the discount which was given to the patient at the point-of-care at the Clinical Encounter (claim item level). If no discount was given, then this element will indicate zero Rands.	Both	M	M	M	M
Calculated Item Discount Amount		The Processing Intermediary indicates the discount which the Billing Practice should have extended at the point-of-care typically based on defined business rules or contractual agreements. CM: This element is mandatory if it is required to determine the Approved Discount for Electronic Claims.	Electronic	CM	CM	CM	CM
Approved Item Discount Amount		The Processing Intermediary determines this amount as a function of the Submitted and/or Calculated Discount based on the Payor's business rules. For example, the Approved Discount could be minimum (or maximum) of the Submitted and Calculated Discounts. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Submitted Pricing Tariff Indicator	2N	This is an attribute of the Submitted Base Cost element and is used by the **Billing Practice** to indicate the pricing model used to determine the base cost. The element is mandatory if required by the Processing Intermediary or Payor. Refer to Table 9.	Both	M	M	M	M
Calculated Pricing Tariff Indicator		This is an attribute of the Calculated Base Cost element and is used by the Processing Intermediary to indicate the pricing model used to determine the base cost. This attribute is mandatory when the Calculated Base Cost element is provided. Refer to Table 9. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Approved Pricing Tariff Indicator		This is an attribute of the Approved Base Cost element and is used by the Processing Intermediary to indicate the pricing model used to determine the base cost. This attribute is mandatory when the Approved Base Cost element is provided. Refer to Table 9. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Submitted Item Collection Amount	9N RRRRRRR.CC	The billing practice specifies the amount(s) actually collected from the patient or member or intended to be collected (i.e. a debtor) per line item. The element is always mandatory and must indicate zero Rand if no collection was made or is intended to be made.	Both	M	M	M	M
Calculated Item Deduction Amount		This is the value which the processing intermediary deems the billing practice should have collected from the patient at the point-of-care per line item. The processing intermediary uses this data element to explain the breakdown of the Calculated Item Deduction Amounts and is therefore only mandatory if required by the Processing Intermediary or Payor. Refer to Deduction Type data element. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Approved Total Deduction Amount		The payor determines the amount as a function of the Submitted Item Collection and/or Calculated Item Deduction based on the payor's business rules. CM: Electronic Claims only. To illustrate the usage, this could either be : • Equal to the Submitted Item Collection, the billing practice accepts the upfront item collection and the payor will settle differences directly with the member, or • Equal to the Calculated Item Deduction, the processing intermediary enforced the calculated item deduction amount and expects the billing practice to settle differences with the member, or • In the case where a patient submits a claim and not the billing practice, the value could be equal to the difference between the Approved Gross Item Amount MINUS Approved Item Discount Amount.	Electronic	CM	CM	CM	CM
Item Deduction Type Indicator	2N	Refer to Table 14. CM: This is an attribute of Deductions and is mandatory if Deductions are included in the message for Electronic Claims. Deduction Type is enumerated as follows: • Levy: Also known as a deductible, the levy is a fee determined by the Payor which is due by the member/patient to the Billing Practice at the point-of-care. As a demand-side control of expenditure through an out-of-pocket expense levied on the member/patient, it is typically a fixed Rand or percentage value calculated at claim or item level according to the Payor's registered rules. The levy amount is typically not refunded from the Payor's risk benefit but might be reimbursed from the member's savings benefit. • Co-payment: Similar to a Levy, however it is a variable fee as a function of factors that the Processing Intermediary is privy to; such as patient benefits available, rules unique to a particular patient / service / product combination. For example, in the case where the Billing Practice can legitimately claim R65 from the Payor, but where the patient runs out of benefit or a limit applies to the extent that there is a R5 shortfall; then the Co-Payment would be R5.	Electronic	CM	CM	CM	CM

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Sum of Line Item's Calculated Deduction Amount	9N RRRRRRR.CC	The processing intermediary determines the sum of each line item's separate Calculated Item Deduction Amounts. The 'Sum of Line Item's Calculated Deduction Amount' element cannot be repeated for a single line item. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Modifier Amount	9N RRRRRRR.CC	The modifier amount could be specified for information / clarification purposes. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM

Note:

- Currency symbols to be omitted on both paper and electronic claims.
- All values quoted will be VAT inclusive.
- Table 9: Pricing Tariff Indicators

01	NHRPL Rate
02	Private Tariff Rate
03	Medical Scheme Rate
04	Contractual Rate
05	Manufacturer's Price
06	COID Rate
07	Ethical Tariff Rates
08	Unknown
09	Single Exit Price (SEP)
10	Nett Acquisition Price (NAP)

- Table 14: Deduction Type Indicators

01	Levy
02	Co-payment

- Table 15: Product Coding Standard Indicators

01	NAPPI
02	SAOA
03	SAOPA
04	Proprietary Code Set

2.3.20 System Information

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Operating System	50AN	Name of originator's operating system. Mandatory for electronic Claims. CM for paper Claims with the proviso that the system producing the paper Claim has the ability to generate the information.	Both	CM	CM	CM	CM
Source Identifier	15AN	Originator's unique reference to database or machine identifier. This field could be used by the PMA to reconcile any messages received from other parties in the communication channel. Mandatory for electronic Claims. CM for paper Claims with the proviso that the system producing the paper Claim has the ability to generate the information.	Both	CM	CM	CM	CM
PMA Title	50AN	The Practice Management Application's (PMA) name and not the name of the company owning / developing the software product. Mandatory for electronic Claims. CM for paper Claims with the proviso that the system producing the paper Claim has the ability to generate the information.	Both	CM	CM	CM	CM
PMA Version Number	10AN	The version number of the Practice Management Application (PMA) installed at the billing practice with which Claims are created and submitted. Mandatory for electronic Claims. CM for paper Claims with the proviso that the system producing the paper Claim has the ability to generate the information.	Both	CM	CM	CM	CM

Note:

- Similar to the transaction path tracking information, certain system information of the Healthcare Service Provider's PMA system would greatly support fault finding and channel optimisation. The claim message allows for the population of certain system information. An example is the source ID which is a unique reference in the Healthcare Service Provider's PMA system to indicate on which workstation/device the claim was captured.

- The business rule is that if this information is populated, the exact same information must be echoed back in the response message.

2.3.21 XML Technical Implementation Information

The technical implementation of this standard makes use of collections of data. Information is stated once with a unique identifier and instead of duplicating the information, the unique identifier is used throughout the XML message. Benefits of using this method are that duplication is reduced, message sizes are reduced and re-usability is increased. This concept was used for all person entities in the schema.

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Non-provider Trading Party ID	2N	Unique identifier used to refer to a single trading party in a list of non-provider trading parties. A non-provider trading party is any trading party in the channel that did not create the Claim / Credit Note. Integer starting at 1 incrementing by 1 for every new entry in the list. CM: Electronic Claims only. Refer to Table 21.	Electronic	CM	CM	CM	CM
Treating Provider ID	2N	Unique identifier used to refer to a treating provider in a list of treating providers. Refer to section 2.3.5 for more detailed information regarding Provider Information. Integer starting at 1 incrementing by 1 for every new entry in list. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Referring Provider ID	2N	Unique identifier used to refer to a referring provider OR prescribing provider in a list of referring / prescribing providers. Refer to section 2.3.7 for more detailed information regarding Referral and Prescriber Information. Integer starting at 1 incrementing by 1 for every new entry in list. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Assisting Provider ID	2N	Unique identifier used to refer to a single assisting provider in a list of assisting providers. Refer to section 2.3.16 for more detailed information regarding Item Assistant Information. Integer starting at 1 incrementing by 1 for every new entry in list. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Dental Technician Laboratory ID	2N	Unique identifier used to refer to a single dental laboratory in a list of dental laboratories. Refer to section 2.3.12.2 for more detailed information regarding Additional Information for Dental Laboratory items Integer starting at 1 incrementing by 1 for every new entry in list. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Patient ID	2N	Unique identifier used to refer to patient information. Refer to section 2.3.8 for more detailed information regarding Patient Information. Only one entry is allowed in this collection. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Diagnosis ID	2N	Unique identifier used to refer to a group of diagnosis codes. Integer starting at 1 incrementing by 1 for every new entry in list. This could be referring or treating diagnosis codes. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Financial Element ID	2AN	This is a mandatory attribute of every financial element and is used in other parts of the same message to reference specific financial elements; e.g. Financial Notes structure CM: Electronic Claims only.	Electronic	CM	CM	CM	CM

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Modifier Sequence Number	2N	This Sequence number is used to indicate the order in which the modifiers should be applied. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM

Note:

- Table 16: Non-provider Trading Party Identifiers

PHISC Identifier	Non-Provider Trading Party's Name
EMD001	eMD
SWI001	DH Switch
GDX001	GoodX
HBR001	HealthBridge
LAN001	Lancet

2.3.21.1 XML Financial Note Information

The XML schema makes provision for non-core Information Financial Fields. The purpose of these fields is to allow trading parties to explain and break down those fields describe in the Financial Information sections using a more free-form approach. The structure, called Financial Notes, is provided at a header level in the XML message and allows for any multiple repetitions of the following atomic structure to be added by any trading party in the channel:-

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Financial ID	2N	Unique identifier used in the technical implementation of this standard to refer to a note in a list of financial notes. Integer starting at 1 incrementing by 1 for every new entry in list. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Financial Note Code	5AN	This is an optional financial note code which may be used between parties to uniquely encode the description provided and are agreed between the parties as necessary. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Financial Note Description	100AN	A free-text description of the financial note being added. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM
Financial Note Amount	9N RRRRRRR.CC	If the description of the financial note requires a value, then this field would be used to indicate the amount. The unit of measure is specified in the description. CM: Electronic Claims only.	Electronic	CM	CM	CM	CM

2.3.21.2 XML Envelope Information

Purpose of the envelopes is to contain information to enable:-

- Transmission management i.e. status of transaction.
- Routing management i.e. who submitted the message and any previous routing information.
- Integrity of the information through the use of control totals.
- Confidentiality of the sensitive information can be preserved in the payload, whilst making available non-sensitive routing information in the envelope.

The PHISC standard makes provision for envelopes at two levels:-

- Batch: This is applied to batch or a collection of claims, responses or remittance advices intended for a specific recipient.
- Message: The main purpose of this envelope is to ensure confidentiality of information when encrypted.

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Sending Trading Party Batch Number	20AN	This unique number is generated by any trading party in the channel who creates the batch and is used by recipients to detect duplicate batches.	Electronic	CM	CM	CM	CM
Date / Time Tracking Information Added	8N Date format CCYYMMDD 5AN HH:MM	Date / Time that the tracking information was added to the envelope.	Electronic	CM	CM	CM	CM
Date / Time Transmitted	8N Date format CCYYMMDD 5AN HH:MM	Date / Time that the message was transmitted to the next trading party.	Electronic	CM	CM	CM	CM
Date / Time Received	8N Date format CCYYMMDD 5AN HH:MM	Date / Time that the message was received.	Electronic	CM	CM	CM	CM
Number of Items in Batch	5N	The number of items in the batch. This could be Claims, Amended Claims or Credit Notes.	Electronic	CM	CM	CM	CM
Submitted Claim Total Amount	9N RRRRRRR.CC	The total submitted claimed amount that is used for reconciliation purposes of a Claim.	Electronic	CM	CM	CM	CM
Approved Claim Amount Total	9N RRRRRRR.CC	The total approved claimed amount that is used for reconciliation purposes of a Response message.	Electronic	CM	CM	CM	CM
Corrupt Batch Continue on Error	1AN	This request indicator will indicate whether the billing practice would prefer that the Processing Intermediary reject the entire batch if one message fails or to process on an individual message basis. Note that this indicator represents a request only. The Processing Intermediary may still process the batch differently.	Electronic	CM	CM	CM	CM
Honoured Batch Continue on Error	1AN	This request indicator is used in the Response message and will indicate whether the 'Corrupt Batch Continue On Error' request in the Claim message was honoured or not. TRUE will indicate that the request was honoured and FALSE will indicate that the request was not honoured.	Electronic	CM	CM	CM	CM

2.3.21.3 XML Response Message Information

It is the intention of this Forum to define a standardised list of commonly used Reason Codes. This list of PHISC Reason Codes will be published as part of a future release of this Content document. At present, Trading Parties will use their own set of proprietary Reason Codes and their descriptions.

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Claim / Invoice / Credit Note Response Number	35AN	Unique number generated by the Medical Scheme for each Response message generated. Number must be globally unique by using the Medical Scheme Registration Number (section 2.3.3) + Response number. CM: Electronic only	Electronic	CM	CM	CM	CM
Response Disclaimer	50AN	Description CM: Electronic only	Electronic	CM	CM	CM	CM
Response Tracking Status	20AN	This field indicates the stage of processing of the Claim or Credit Note once a Trading Party has validated and / or processed the message and generated a Response. Refer to Table 16. CM: Electronic only	Electronic	CM	CM	CM	CM
Response Process Status	10AN	The Response message's Process Status for a specific item in a Claim or Credit Note is indicated. Refer to Table 17. CM: Electronic only	Electronic	CM	CM	CM	CM
Response Reason Code Sequence Number	5N	The submission sequence of the Response message's Reason Code. CM: Electronic only	Electronic	CM	CM	CM	CM
Response Reason Code Type	10AN	This information indicates to the billing practice / supplier whether any actions are required by them or not. Refer to Table 18. The information could be used by the billing practice / supplier's PMA software in order to assist the billing practice's business workflow processes. CM: Electronic only	Electronic	CM	CM	CM	CM
Response Reason Code	10AN	A unique, proprietary code assigned by the trading party and linked to a Reason Response Code Description. CM: Electronic only	Electronic	CM	CM	CM	CM
Response Reason Code Description	50AN	A proprietary description of the error, warning and / or information supplied by the trading party and linked to a Reason Response Code. CM: Electronic only	Electronic	CM	CM	CM	CM

Field Name	Data Format & Length	Field Description	Claim Submission Requirement	Practice Type Claims Submission			
				General Practitioners & Specialists & Auxiliaries	Pharmacy	Optometry	Pathology
Response Reason Code Source Indicator	1AN	To make provision for the future implementation of a standardised Response Reason Code list, the Response Reason Code Source Indicator will indicate the source of the Reason Code. Refer to Table 19. CM: Electronic only	Electronic	CM	CM	CM	CM
Reason Code Type Qualifier	2AN	The Reason Code Type Qualifier field will be used to distinguish between clinical messages and purely informational messages received from processing parties in order to assist PMA software vendors with workflow processes. The Response Reason Code Type Qualifiers are typically dealt with by IT systems / vendors and not the practice / provider. Refer to Table 22 CM: Depends on the processing and receiving parties' capability of handling such information as well as only applicable to the electronic Response Message.	Electronic	CM	CM	CM	CM
Advisory Action Indicator	2AN	Only one (1) Advisory Action Indicator per Reason Code Type Qualifier is allowed. Refer to Table 23. CM: Depends on the processing and receiving parties' capability of handling such information as well as only applicable to the electronic Response Message.	Electronic	CM	CM	CM	CM

Note:

- Table 19: Response Reason Code Source Indicators

TRUE	PHISC as source
FALSE	Other party as source

- Table 16: Response Tracking Status Indicators

Response Tracking Status Indicators	Response Tracking Status Description
Received	As per the definition of a Confirmation of Receipt Response message: The message was parsed, complies with the PHISC XML schema and persisted to a non-volatile storage device and has been accepted for further processing.
Switched	The message was 'Received' and passed onto another trading party for further processing.
Partly Processed	The message was partly processed and the Response message contains the result of this processing, but that further Response messages should be expected from this or other trading parties.
Fully Processed	This message was fully processed and the Response message contains the final result of processing. The Processing Intermediary of this result assures that no further Response messages should be expected on this message from any other parties in the trading community.
Failed	The recipient of the message was unable to validate or process the message for reasons indicated in the Response message.

- Table 17: Response Process Status Indicators

Response Process Status Indicators	Response Process Status Description
Received	The item was parsed, complies with the PHISC XML schema and persisted to a non-volatile storage device.
Valid	The item conforms to the mandatory information and data integrity requirements of the recipient.
Invalid	The item is missing mandatory information or contains invalid information according to the mandatory information and data integrity requirements of the recipient.
Rejected	The item has been successfully processed BUT the Claim item / Credit Note item has been rejected and no payment for this item will be made.
Adjusted	The item has been successfully processed BUT although payment has been adjudicated for the item, payment will be less than the Claimed Amount.
Pended	The item has been Received but is still being processed. No further information is available at the time of generating the Response message,
Referred	The item has been Received but has been referred to another party for processing. No further information is available at the time of generating the Response message,
Approved	The item has been successfully processed AND the amount claimed has been approved for payment as requested.
Paid	The item has been successfully processed AND paid. This status is expected to be restricted for use in the Electronic Remittance Advice.

- Table 18: Response Reason Code Type Indicators

Response Reason Code Value	Response Reason Code Description
Rejection	The Claim / Credit Note will not be processed further as a consequence of the reason provided.
Warning	The Reason Code and description has a material impact on the processing and payment of the Claim. There is likely to be a difference between the Billing Practice / Supplier's expectation of payment and the final adjudication.
Information	The Reason Code and description provides additional information which the Billing Practice / Supplier should take cognisance of.
Unknown	In instances where a processing intermediary is not able to determine the Response Reason Code Type Indicator, 'Unknown' should be used.

- Table 22: Reason Code Type Qualifiers

Qualifier	Short Description	Long Description
01	Invalid / Missing Policy Data	Invalid or missing policy information, suspensions, re-activations, resignations, option codes, invalid funder data.
02	Invalid / Missing Practice or Provider Data	Invalid or missing service provider data, including referring, attending, billing, prescribing providers.
03	Invalid / Missing Coding Data	This code doesn't exist in the world. Including but not limited to procedural, NAPPI, ICD-10, Place of Service codes and any other codes.
04	Invalid Message Integrity	Control totals, missing mandatory information, invalid indexing, incorrect financial totals, claim line counts. This item is typically dealt with by IT systems / vendors and not the practice / provider.
05	Funder Rules	Benefit exclusions, financial exclusions, Designated Service Providers (DSPs), invalid / missing authorisation number, invalid service dates, and outright rejections.
06	Inappropriate Clinical Application	Quantity, clinical codes like tariff, DUR reason codes.
07	Technical Error	Any party in the channel is offline, communication failure / fault / outage.

- Table 23: Advisory Action Indicators

Qualifier	Short Description	Long Description
01	No Action	No action required.
02	Verify and / or correct data: Reversal	Submit an Amended Claim and / or Credit Note.
03	Verify and / or correct data: Resend	Submit a Copy Claim
04	Member Liable	-
05	Destination not available, try again later	-
06	Refer to reason code for action	-

2.4 Modifier Claim Submission Standardisation

2.4.1 Overview

- The PHISC Standardisation of Claim Content Subcommittee discussed extensively over several workshops the various claiming models and motivations for the standardisation for the submission of modifiers. This proposal will aim to document the various models discussed as well as the final proposal that was accepted by the subcommittee.

- The subcommittee consists of PMA (Practice Management Application) software vendors, Medical Schemes, Providers of Healthcare, BHF, SAMA and third party switching vendors.

2.4.2 Objectives of the workshops:

- The intent of the various workshops was not to discuss the validity of the modifiers but rather to determine and propose a standardised claiming format.

- The solution to be provided by the software (PMA) system and utilized by the Provider when posting and submitting claims containing modifiers to Medical Schemes.

- Workshops were held on the following dates and reference to the minutes of the meetings:
    - 18/02/2005 at Discovery Health
    - 07/04/2005 at Medscheme
    - 28/04/2005 at Old Mutual
    - 14/06/2005 at Discovery Health

2.4.3 Industry problems with modifier claim submission:

- There is lack of standards in billing, processing and interpretation of modifiers.
- A separate work group is required to define and interpret modifiers.
- Providers bill modifiers to their benefit.
- Cost to the scheme due to the differences.
- Specialist anaesthetist packages seldom quote modifier 0023 as they feel it is implicit. Because it is not quoted on the bill it is up to the Medical Scheme to determine what the providers' intentions are.
- Modifier 0023 is a time-based modifier and it is critical that the modifier must be submitted.
- There is some confusion over the various categories of modifiers; which ones adds value, subtracts value and serves as informational modifiers.
- Due to the flexibility within the PMA systems – the providers can elect various ways to submit modifiers to the Medical Schemes and hence there is no standard.
- With modifiers being billed at the end of the claim or on separate lines, there is uncertainty as to which tariff codes the modifiers relate to and in which sequence they apply.
- As a result of the above, automated pricing becomes very difficult.
- Medical Practitioner Modifier 0005 is applied and not quoted.
- Medical Schemes process in the sequence the modifiers are submitted.

We would like to thank SAMA for granting us permission to use their modifier descriptions published in the Medical Practitioners' section of this document. Please take note that the full SAMA descriptions have not been used, but rather an abbreviated version. For full modifier descriptions, please consult the SAMA Doctors' Billing Manual (DBM). All other modifier descriptions are obtained from the National Health Reference Price List (NHRPL). Please take note that the modifier descriptions published in this document may change without prior notification. Please consult the relevant SAMA or NHRPL publications for up to date information.

2.4.4 Classification of Modifiers

A modifier is defined as a code that is used to modify / alter / amend a line item and / or a Claim and / or a portion of a Claim. No modifier can be used on its own but must ALWAYS be used in conjunction with a procedural code and / or other modifiers.

Informational Modifiers

When a modifier serves to add additional information to a procedural code and has no financial value.

Reduction Modifiers

When a modifier reduces the financial value, by using a percentage value or a unit value, of a procedure code.

Add Modifiers

When a modifier adds financial value, by using a percentage value or a unit value, to a procedure code.

Compound Modifiers

Modifiers that serve to both add information and financial value by using a percentage value or a unit value, to a procedure code and / or a Claim and / or a portion of a Claim. The financial value of the modifier is then a value of a function of certain values in a Claim applied to one or many item lines.

2.4.5 Modifiers Submission Models

The following models were discussed extensively:

Model	Description	Advantages	Disadvantages
1.	Modifier and Tariff billed in the same line with a single value	• Billing and Reconciliation is easier • Printed statement – less paper is required • Eliminates the problem of interpretation.	• The maximum number of modifiers applied to a single tariff is estimated at 7. • Difficulty in determining where the excess billing takes place.
2.	Modifier and Tariff billed with separate values on one line	• Billing and Reconciliation is easier • Printed statement – less paper is required • Eliminates the problem of interpretation.	• The maximum number of modifiers applied to a single tariff is estimated at 7. • Difficulty in determining where the excess billing takes place.
3.	All Modifiers and Tariffs billed with separate values on separate lines	• Intent of billing can be viewed. • Excess billing can be determined • Easier on paper claims to read.	• Linking and sequence of the modifiers is implied. • Not all systems can deal with a negative or zero value.
4.	Modifier and Tariff billed on the same line with a single value, with specific modifiers billed on separate lines with their own separate values.	• Intent of billing can be viewed. • Excess billing can be determined • Easier on paper claims to read.	• Linking and sequence of the modifiers is implied.

2.4.6 Business Rules and Assumptions

- The method used to claim any modifier is documented in more detail under each submission method's layout requirements. For paper submissions, refer to section 3 Guidelines for Printed Paper Claim Forms in this document and for electronic submission, refer to the PHISC XML Transaction Specification document.
- Reduction modifiers are applied first and then the ADD modifiers are applied to the reduced value.
- A modifier must be claimed to be paid and is not implicit.
- A suggestion is for the Council of Medical Schemes (CMS) to publish as part of the NHRPL guidelines as a best practice the modifier categories and defined rules for processing claims containing modifiers.
- For NHRPL 2006 – to classify modifiers into ADD, Reduce and Informational and Compound (mixed).
- Note that each Assistant's PCNS number is accompanied per line that will indicate that there is not a duplicate on the modifier 0009 or 0008 as the PCNS numbers are different in the case of multiple assistants.
- Reconciliation with the Supplier / Billing Practice will be exactly as the lines were submitted i.e. the Medical Scheme will reconcile with the Supplier / Billing Practice and match line for line.
- ICD10 codes are required for the tariff lines but not for the individual modifier lines (where applicable).
- Modifier codes that indicate that a minimum rule must be applied are considered ADD modifiers, for example Medical Practitioner modifier 0035.

2.4.7 Rounding Methods

The following rounding methods must be used when modifiers are quoted:

- NHRPL: When ADD, REDUCTION and / or COMPOUND modifiers are quoted, the total value or the modifier's value must be rounded off to the nearest ONE cent.
- COID: When ADD, REDUCTION and / or COMPOUND modifiers are quoted, the total value or the modifier's value must be rounded off to the nearest 10 cents.

2.4.8 Modifiers per Discipline

The modifier listing is based on the 2006 NHRPL publication due to the fact that no 2007 NHRPL has been published.

2.4.8.1 Clinical Technology

Modifier	Info	Reduce	Add	Compound	Modifier Shortened Description
0001			✓		Fee prorated according to number of treatment days; fee = ([number of treatment days] / 30) x (item fee).

2.4.8.2 Dental Practitioners

Modifier	Info	Reduce	Add	Compound	Modifier Shortened Description
8001				✓	Assistant surgeon - specialist: 1/3 of the appropriate benefit
8002			✓		Specialist fee / benefit: Plus 50% of the appropriate benefit
8006		✓			Multiple surgical procedures - third and subsequent procedures (50% of the appropriate benefit)
8007				✓	Assistant surgeon - general dental practitioner (15% of the appropriate benefit)
8008			✓		Emergency surgery - after hours (PLUS 25% of the appropriate benefit)
8009		✓			Multiple surgical procedures - second procedure (75% of the appropriate benefit)
8010			✓		Open reduction (PLUS 75% of the appropriate benefit)
8011			✓		Procedure accompanied by unusual circumstances (Benefit PLUS X % as determined by the practitioner and agreed upon by patient/medical scheme)
8012		✓			Reduced services (benefit MINUS X % as determined by the practitioner)
8013	✓				Multiple modifiers
8023			✓		Fabrication of inlay/onlay (PLUS 25% of the appropriate benefit)

2.4.8.3 Dieticians

Modifier	Info	Reduce	Add	Compound	Modifier Shortened Description
0021	✓				Services to hospital inpatients: Quote modifier 0021 on all accounts for service performed on hospital inpatients.

2.4.8.4 Medical Practitioners

Modifier	Info	Reduce	Add	Compound	Apply to[3]	Modifier Shortened Description
0001			✓		N/A	**COID only:** After-hours emergency radiological services
0002	✓				N/A	Written report on X-rays
0004			✓		N/A	Procedures performed in own procedure rooms
0004					N/A	Applicable to Compensation Commissioner cases only: Modifier 0004: NOT APPLICABLE TO COMPENSATION COMMISSIONER CASES
0005		✓			N/A	Multiple therapeutic procedures / operations under the same anaesthetic Refer to Note 1 for additional information.
0006	✓				N/A	Visiting specialists performing procedures
0006		✓			N/A	Applicable to Compensation Commissioner cases only: Visiting specialists performing procedures
0007			✓		N/A	(a) Use of own monitoring equipment in the rooms. (b) Use of own equipment in hospital theatre or unattached theatre unit.

Modifier	Info	Reduce	Add	Compound	Apply to[3]	Modifier Shortened Description
0008				✓	N/A	Specialist surgeon assistant
0009				✓	N/A	Assistant
0010			✓		N/A	Local anaesthetic
0011				✓	N/A	Emergency procedures
0011				✓	N/A	Applicable to Compensation Commissioner cases only: Emergency surgery for theatre procedures Refer to Note 6 for additional information.
0013		✓			N/A	Endoscopic examinations done at operations
0014	✓				N/A	Operations previously performed by other surgeons
0015	✓				N/A	Intravenous infusions
0017		✓			N/A	Injections administered by practitioners Refer to Note 7 for additional information.
0018			✓		0023	Surgical modifier for persons with a BMI of 35>
0019			✓		0023	Surgery on neonates (up to and including 28 days after birth) and low birth weight infants (less than 2500g) under general anaesthesia
0020	✓				Proc	Conscious sedation
0021	✓				0023	Determination of anaesthetic fees
0023			✓		N/A	Anaesthetic Refer to Note 4, 6 for additional information.
0024	✓				Proc	Pre-operative assessment not followed by procedures
0025	✓				0023	Calculation of anaesthetic time
0027	✓				Proc	More than one procedure under the same anaesthetic
0028	✓				0023	Indicator for use of low flow anaesthetic technique less than 1 litre / minute
0029				✓	N/A	Assistant anaesthesiologists
0030	✓				0023	Indicator for use of low flow anaesthetic technique less than 1 - 2 litres / minute
0031	✓				Proc	Intravenous drips and transfusions
0032			✓		Proc	Patients in prone position
0033	✓				Proc	Participating in general care of patients
0034			✓		Proc	Head and neck procedures
0035			✓		0023	Anaesthetic administered Refer to Note 6 for additional information.
0036		✓			0023	Anaesthetic administered by general practitioners Refer to Note 2, 5, 6 for additional information.
0037			✓		Proc	Body hypothermia
0038			✓		Proc	Peri-operative blood salvage
0039			✓		Proc	Control of blood pressure
0040			✓		Proc	Phaeochromocytoma
0041			✓		Proc	Hyperbaric pressurisation
0042			✓		Proc	Extracorporeal circulation
0043			✓		Proc	Patients under one year of age
0044			✓		Proc	Neonates (i.e. up to and including 28 days after birth)
0045	✓				Proc	Post-operative alleviation of pain

Modifier	Info	Reduce	Add	Compound	Apply to[3]	Modifier Shortened Description
0046		✓			N/A	Where in the treatment of a specific fracture or dislocation (compound or closed) an initial procedure is followed within one month by an open reduction, internal fixation, external skeletal fixation, or bone grafting on the same bone, the fee for the initial treatment of that fracture or dislocation shall be reduced by 50%. Please note: This reduction does not include the assistant's fee where applicable. After one month, a full fee as for the initial treatment is applicable.
0047	✓				N/A	A fracture NOT requiring reduction shall be charged on a fee per service basis.
0048			✓		N/A	Where in the treatment of a fracture or dislocation, an initial closed reduction is followed within one month by further closed reductions under general anaesthesia, the fee for such subsequent reductions will be 27.00 clinical procedure units (not including after-care)
0049			✓		N/A	Compound fractures
0050			✓		N/A	Compound fracture where a debridement is followed by internal fixation
0051			✓		N/A	Fractures requiring open reduction, internal fixation, external skeletal fixation and / or bone grafting
0053			✓		N/A	Fracture requiring percutaneous internal fixation [insertion and removal of fixatives (wires) in respect of fingers and toes included]
0055			✓		N/A	Dislocation requiring open reduction
0057		✓			N/A	Multiple procedures on feet
0058			✓		N/A	Revision operation for total joint replacement and immediate re-substitution (infected or non-infected)
0061	✓				N/A	Combined procedures on the spine
0063		✓			N/A	Two specialists work together on a replantation procedure
0064	✓				N/A	Replantation is unsuccessful
0065		✓			N/A	Additional operative procedures by same surgeon within a period of 12 months
0066			✓		N/A	Microsurgery of fallopian-tubes and ovaries
0067			✓		N/A	Microsurgery of larynx
0069			✓		N/A	When endoscopic instruments are used during intranasal surgery
0070			✓		N/A	Procedures performed through thorascope
0072	✓				N/A	Non-invasive peripheral vascular tests
0073			✓		N/A	When item 1288 or item 1289 is performed by paediatric cardiologists
0074			✓		N/A	Endoscopic procedures performed with own equipment
0075			✓		N/A	Endoscopic procedures performed in own procedure room
0077	✓				N/A	Physical treatment
0078			✓		N/A	When a testis biopsy is combined with vasogram, seminal vesiculogram or epididymogram
0079	✓				N/A	When a first consultation / visit proceeds into or is immediately followed by a medical psychotherapeutic procedure
0080	✓				N/A	Multiple examinations
0081	✓				N/A	Repeat examinations
0082	✓				N/A	'+' means that this item is complementary to a preceding item and is therefore not subject to reduction
0083		✓			N/A	Radiological examinations

Modifier	Info	Reduce	Add	Compound	Apply to[3]	Modifier Shortened Description
0084			✓		N/A	Film costs
0085	✓				N/A	Left side modifier
0086	✓				N/A	Vascular groups
0090				✓	N/A	Radiologist's fee for participation in a team
0091	✓				N/A	Diagnostic services rendered to hospital in-patients
0092	✓				N/A	Diagnostic services rendered to out-patients
0095	✓				N/A	Radiation materials
0096	✓				N/A	Radio-isotope therapy patients who fail to keep their appointments
0097	✓				N/A	Pathology tests performed by non-pathologists
0099			✓		N/A	Stat basis tests
0100			✓		Proc	Intra-aortic balloon pump
0160			✓		N/A	Aspiration of biopsy procedure performed under direct ultrasound control by an ultrasound aspiration biopsy transducer
0165			✓		N/A	Use of contrast during ultrasound study
5104			✓		N/A	Ultrasound in pregnancy, multiple gestation, after twenty weeks
5441			✓		Proc	Add one (1.00) anaesthetic unit; except where the procedure refers to the bones named in Modifiers 5442 to 5448.
5442			✓		Proc	Shoulder; scapula; clavicle; humerus; elbow joint; upper 1\3 tibia; knee joint; patella; mandible and temperomandibular joint
5443			✓		Proc	Maxillary and orbital bones
5444			✓		Proc	Shaft of femur
5445			✓		Proc	Spine (except coccyx); pelvis; hip; neck of femur
5448			✓		Proc	Sternum and\or ribs and musculo-skeletal procedures which involve an intra-thoracic approach
6100	✓				N/A	In order to charge the full fee (600.00 Magnetic Resonance units) for an examination of a specific single anatomical region, it should be performed with the applicable radio frequency coil including T1 and T2 weighted images on at least two planes.
6101		✓			N/A	Where a limited series of a specific anatomical region is performed (except bone tumour); e g a T2 weighted image of a bone for an occult stress fracture; not more than two-thirds (2\3) of the fee may be charged. Also applicable to all radiotherapy planning studies; per region.
6102		✓			N/A	All post-contrast studies (except bone tumour); including perfusion studies; to be charges at 50% of the fee.
6103	✓				N/A	Post-contrast study
6104		✓			N/A	Limited examination of the hypophysis e.g. where a coronal T1 and sagittal T1 series are performed; two-thirds (2\3) of the fee is applicable.
6105	✓				N/A	Where, in a limited hypothesis examination, Gadolinium is administered and coronal T1 and sagittal T1 series are repeated, a single full fee for the entire examination is applicable + cost of Gadolinium + disposable items.
6106	✓				N/A	Where a Magnetic Resonance Angiography (MRA) of large vessels is performed as primary examination, 100% of the fee is applicable. This modifier is only applicable if the series is performed by use of a recognised angiographic software package with reconstruction capability.

Modifier	Info	Reduce	Add	Compound	Apply to[3]	Modifier Shortened Description
6107		✓			N/A	Where a Magnetic Resonance Angiography (MRA) of the vessels is performed additional to an examination of a particular region; 50% of the fee is applicable for the angiography. This modifier is only applicable if the series is performed by use of a recognised angiographic software package with reconstruction capability.
6108		✓			N/A	Where only a gradient echo series is performed with a machine without a recognised angiographic software package with reconstruction ability; 20% of the full fee is applicable specifying that it is a flow sensitive series.
6109		✓			N/A	Very limited studies to be charged at 33.33% of the full fee e.g. MR urography for renal colic; diffusion studies of the brain additional to routine brain.
6110		✓			N/A	Magnetic Resonance Imaging (MRI) spectroscopy: 50% of fee
6300		✓			N/A	If a procedure lasts less than 30 minutes, only 50% of the machine fees for items 3536 – 3550 will be allowed.
6301		✓			N/A	If a procedure is performed by a radiologist in a facility not owned by himself, the fee will be reduced by 40%
6302		✓			N/A	When a procedure is performed by a non-radiologist, the fee will be reduced by 40%
6303		✓			N/A	When a procedure is performed entirely by a non-radiologist in a facility owned by a radiologist, the radiologist owning the facility may charge 55% of the procedure units used.
6305		✓			N/A	When multiple catheterisation procedures are used and an angiogram investigation is performed at each level, the unit value of each such multiple procedure will be reduced by 20.00 radiological units for each procedure after the initial catheterisation.

Note 1: Modifier 0005

Modifier 0005 used to indicate that multiple therapeutic procedures / operations were done under the same anaesthetic MUST be quoted on ALL the procedures where it is applied. This includes the first or major procedure.

Note 2: Modifier 0036

Modifier 0036 is only used by general medical practitioners when the administered anaesthetic lasts more than one hour in order to reduce the fee to 80%. The total anaesthetic units are calculated by adding the basic anaesthetic units, the time units and any other appropriate modifiers. This means that modifier 0036 must be applied to the basic anaesthetic units, the time units and any other appropriate modifiers.

Note 3: Apply to column

This column indicates how the NHRPL anaesthetic modifiers should be applied. Some of these modifiers influence the basic anaesthetic units and others only apply to the time units. Modifiers with Proc. indicate that the modifier should be quoted together with the procedure code containing the basic anaesthetic units and 0023 indicates that these modifiers should be quoted together with modifier 0023. For example:

	Procedure code	: Contains Basic Anaesthetic Units	
+	Modifier rules and information	: Add, Reduce, Informational	: Indicated as Proc
+	Anaesthetic Time Modifier 0023	: Contains Time Units	: Indicated as N/A
+	Modifier rules and information	: Add, Reduce, Informational	: Indicated as 0023

Note 4: Definition of Anaesthesiologist

An anaesthesiologist is a medical doctor that specialized in Anaesthesiology. This practitioner falls in the same category as an orthopaedic surgeon, neurosurgeon or gynaecologist.

Note 5: Definition of Anaesthetist

An anaesthetist is a general medical practitioner that did a post graduate diploma in anaesthetics.

Note 6: Interpreting modifiers 0023, 0035 and 0036

Modifier 0023: Modifier 0023 is applied on BOTH a specialist anaesthesiologist as well as a general anaesthetist's claims.

Modifier 0035: Modifier 0035's description has been changed in the NHRPL for 2006. This modifier now indicates that the minimum anaesthetic units (7.00) are applied on both a specialist anaesthesiologist as well as a general anaesthetist's claims.

Modifier 0036: Modifier 0036's description has been changed in the NHRPL for 2006 to not reflect the minimum anaesthetic units rule (See modifier 0035). Modifier 0036 will ALWAYS be used in conjunction with modifier 0023. This modifier is only used on anaesthetic claims by general medical practitioners that administer anaesthetic with a duration of more than 60 minutes to indicate that a reduction must be made.

Note 6: Applying modifier 0011

Modifier 0011 is a Compound modifier. When a single Claim contains more than one modifier 0011 to indicate Emergency Surgery for multiple members of the surgical team, i.e. surgeon and assistant, two (2) lines must be quoted per member of the surgical team as per the Add modifiers' conventions. The Compound modifiers' convention indicates that modifier 0011 must then also be quoted on the procedure items to indicate which procedures where performed as emergencies. Modifier 0011 only needs to be quoted once per procedure line and does not have to be repeated for each member of the surgical team. Please refer to example 5.1.

Note 7: Applying modifier 0017

Modifier 0017 is still published as a modifier but is in fact interpreted as a procedure item. Modifier 0017 should be claimed as a tariff on its own line. SAMA has published procedure code 0031 in place of modifier 0017. Due to a lack of industry acceptance, the use of modifier 0017 as a procedural code will prevail until further notice. An ICD-10 diagnostic code MUST be quoted when claiming modifier 0017. The following are extracts from the 2008 edition of the SAMA Doctor's Billing Manual:

Modifier 0017: Injections administered by practitioners: When desensitisation, intravenous, intramuscular or subcutaneous injections are administered by the practitioner him- / herself to patients who attend the consulting rooms, a first injection forms part of the consultation / visit and only all subsequent injections for the same condition should be charged according to item 0131 (not chargeable together with a consultation item).
Tariff 0131: Subsequent injections for the same condition administered by practitioners (refer to modifier 0017) (not chargeable together with any consultation item).

Note 8: For examples on how to present modifier claims, please refer to Section D: Basic Guidelines for Printed Paper Standardised Claims: 5. Examples of Modifier Submissions.

2.4.8.5 Occupational & Art Therapy

Modifier	Info	Reduce	Add	Compound	Modifier Shortened Description
006			✓		Add 50% of the total fee for the procedure. Modifier 0006 does not apply to Art therapy
008			✓		Assistive devices to be charged (exclusive of VAT) at net acquisition price PLUS (a) 26% of the net acquisition price where the net acquisition price of that appliance is less than one hundred Rands and (b) a maximum of twenty six Rands where the net acquisition price of that appliance is greater than or equal to one hundred Rands.
009			✓		Materials used for orthoses or pressure garments to be charged (exclusive of VAT) at net acquisition price PLUS (a) 26% of the net acquisition price where the net acquisition price of that material is less than one hundred Rands and (b) a maximum of twenty six Rands where the net acquisition price of that material is greater than or equal to one hundred Rands. See Annexures A & B for non-standard products. Modifier 0009 does not apply to Art therapy.
010			✓		Materials used in treatment to be charged (exclusive of VAT) at net acquisition price PLUS (a) 26% of the net acquisition price where the net acquisition price of that material is less than one hundred Rands and (b) a maximum of twenty six Rands where the net acquisition price of that material is greater than or equal to one hundred Rands.
011			✓		Travelling costs according to AA rates. Please note that although only some medical schemes accept responsibility for the payment of transport expenses, others do so in exceptional cases only.
021	✓				Services rendered to hospital inpatients: Quote modifier 0021 on all accounts for services performed on hospital inpatients.

2.4.8.6 Physiotherapy

Modifier	Info	Reduce	Add	Compound	Modifier Shortened Description
001	✓				Appointment not kept.
003		✓			15% of the relevant rate to be deducted where equipment used is not owned by the practitioner.
006				✓	Add 50% of the total fee for the treatment.
008		✓			Only 50% of the fee for these additional procedures may be charged.
009	✓				The full fee for the additional condition may be charged.
010		✓			Only 50% of the fee for the second condition may be charged.
013			✓		Travelling costs (being more than 16 kilometres in total) according to AA-rate. Please note that although only some medical schemes accept responsibility for the payment of transport expenses, others do so in exceptional cases only.
014	✓				Physiotherapy services rendered to an in-patient in a nursing home or hospital.

2.4.8.7 Podiatry

Modifier	Info	Reduce	Add	Compound	Modifier Shortened Description
0002		✓			For procedures 021 to 031 carried out in a day clinic or unattached operating theatre unit, the rate shall be reduced to two-thirds.
0004	✓				Consultation or treatment in a nursing facility/hospital.
0006	✓				Consultation or treatment at the patient's residence.

2.4.8.8 Private Hospitals

Modifier	Info	Reduce	Add	Compound	Modifier Shortened Description
002			✓		Orthopaedic, Neurosurgical and Vascular: (a) Joint replacements (only hip, knee, shoulder ankle or elbow) (b) Femoral popliteal bypasses (c) Carotid endarterectomies (d) Aortic Aneurysm repair and arterial grafts (e) Neurosurgery (Surgery on the brain and spinal cord only, excludes neurolysis)
003			✓		Cardiac surgery: Cardio-thoracic and Cardio-vascular surgery (a) All open heart surgery, with or without the insertion of a prosthesis, coronary artery bypass grafts and heart transplants. Includes all equipment (except item 513), no additional fees may be charged. NOTE: The above surcharge will also be applicable to approved provincial hospitals.

2.4.8.9 Psychology

Modifier	Info	Reduce	Add	Compound	Modifier Shortened Description
0003			✓		Emergency treatments - Relevant fee plus 50%
0004	✓				Psychology services rendered to an in-patient in a nursing home or hospital.

2.4.8.10 Radiography

Modifier	Info	Reduce	Add	Compound	Modifier Shortened Description
0001			✓		The specified call-out fee may be charged for any bona-fide, justifiable emergency occurring at any hour which requires the practitioner to travel to the patient. Individual medical schemes may require a motivation to accompany the claim.
0021	✓				Services rendered to hospital patients: Quote modifier 0021 on all accounts for services performed on hospital or day clinic patients.
0080	✓				Multiple examinations: Full fees
0081	✓				Repeat examinations: No reduction
0084	✓				Films should be charged under code 300.

2.4.8.11 Radiology

Modifier	Info	Reduce	Add	Compound	Modifier Shortened Description
00091	✓				Radiology and nuclear medicine services rendered to hospital inpatients
00092	✓				Radiology and nuclear medicine services rendered to outpatients
00093		✓			A reduction of one third (33.33%) will apply to radiological examinations where hospital equipment it used

2.4.8.12 Registered Nurses in Private Practice & Nursing Agencies

Modifier	Info	Reduce	Add	Compound	Modifier Shortened Description
0001			✓		Public holidays, add 100%. Nursing agencies only.
0002			✓		Only 50% of the fee in respect of the subsidiary / additional procedures may be changed.
0003	✓				The fee that may be charged in respect of the rendering of a service not listed in this recommended benefit schedule, shall be based on the fee in respect of a fee of a comparable service. Motivation must be attached.
0007		✓			Sundays add 50%. Nursing agencies only.

2.4.8.13 Social Workers

Modifier	Info	Reduce	Add	Compound	Modifier Shortened Description
0003			✓		Add 50% of the total fee for the treatment.
0021	✓				Services rendered to hospital inpatients: Quote modifier 0021 on all accounts for services performed on hospital inpatients.
0022	✓				Services rendered at patients residence: Quote modifier 0022 on all accounts for services performed at the patients residence.

2.4.8.14 Auxiliary Disciplines without Modifiers

- Ambulance Services
- Biokinetics
- Chiropractors
- Dental Therapists
- Genetic Counsellors
- Hearing Aid Acousticians
- Homeopaths
- Hospices
- Medical Technologists
- Mental Health Institutions
- Orthoptists
- Osteopaths
- Pharmacies
- Physical Rehabilitation Hospitals
- Phytotherapists
- Psychometrists
- Speech Therapists & Audiologists
- Sub-Acute Facilities
- Tissue Transportation
- Unattached Operating Theatre Units
- Uniform Patient Fee Schedule

2.4.9 Anaesthetic Claims

- Please take note that all unit values and RCF values as per NHRPL 2006.
- 2006 NHRPL RCF (Rand Conversion Factor) for Anaesthetic is R 31.813
- Examples are used only to explain the principles of applying modifiers not the clinical correctness of a medical claim.

2.4.9.1 Diagram describing Anaesthetic Modifier Application

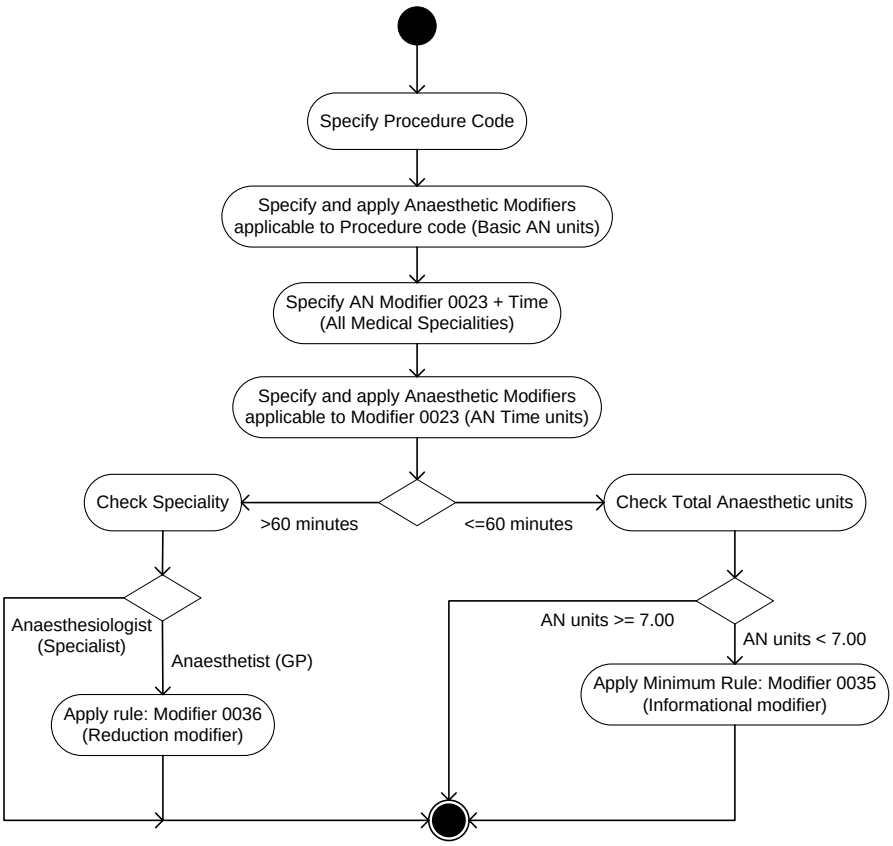

Figure 4: Activity diagram for Anaesthetic Modifier Application

2.4.9.1.a Anaesthesiologist (Specialist) claim with a duration of <= 60 minutes where minimum rule (0035) must be applied:

Duration: 15 minutes and 2006 NHRPL Anaesthetic RCF = R 39.895

Proc. code	Modifier(s)	Qty	Original Anaesthetic Units	Calculated Anaesthetic Units	Calculated Amount	Rounded off Amount	Calculation
0311		1.00	3.00	3.00	R 119.685	R 119.69	Base AN value for 0311
0023	0021 | 0025	15.00	2.00	2.00	R 79.79	R 79.79	Time calculation for 0023
0035		1.00	2.00	2.00	R 79.79	R 79.79	Minimum units rule
TOTALS:			7.00	7.00		R 279.27	

2.4.9.1b Anaesthetist (GP) claim with a duration of <= 60 minutes where minimum rule (0035) must be applied:

Duration: 15 minutes and 2006 NHRPL Anaesthetic RCF = R 39.895

Proc. code	Modifier(s)	Qty	Original Anaesthetic Units	Calculated Anaesthetic Units	Calculated Amount	Rounded off Amount	Calculation
1101		1.00	4.00	4.00	R 159.58	R 159.58	Base AN value for 1101
0023	0021 | 0025	15.00	2.00	2.00	R 79.79	R 79.79	Time calculation for 0023
0035		1.00	1.00	1.00	R 39.895	R 39.90	Minimum units rule
TOTALS:			7.00	7.00		R 279.27	

2.4.9.1.c Anaesthesiologist (Specialist) claim with a duration of > 60 minutes:

Duration: 75 minutes and 2006 NHRPL Anaesthetic RCF = R 39.895

Proc. code	Modifier(s)	Qty	Original Anaesthetic Units	Calculated Anaesthetic Units	Calculated Amount	Rounded off Amount	Calculation
1276		1.00	13.00	13.00	R 518.635	R 518.64	Base AN value for 1276
0023	0021 | 0025	75.00	11.00	11.00	R 438.845	R 438.85	Time calculation for 0023
TOTALS:				24.00		R 957.49	

2.4.9.1.d Anaesthetist (GP) claim with a duration of > 60 minutes where modifier 0036's rule must be applied:

Duration: 75 minutes and 2006 NHRPL Anaesthetic RCF = R 39.895

Proc. code	Modifier(s)	Qty	Original Anaesthetic Units	Calculated Anaesthetic Units	Calculated Amount	Rounded off Amount	Calculation
1109	0036	1.00	6.00	4.80	R 191.496	R 191.50	Base AN value for 1109
0023	0036 | 0021| 0025	75.00	11.00	8.80	R 351.076	R 351.08	Time calculation for 0023 [11.00 AN units] MINUS modifier 0036 [20%]
TOTALS:			17.00	13.60		R 542.58	

2.4.9.1.e Anaesthesiologist (Specialist) claim with a duration of <= 60 minutes where other modifiers must be applied:

Duration: 15 minutes and 2006 NHRPL Anaesthetic RCF = R 39.895

Proc. code	Modifier(s)	Qty	Original Anaesthetic Units	Calculated Anaesthetic Units	Calculated Amount	Rounded off Amount	Calculation
0311		1.00	3.00	3.00	R 119.685	R 119.69	Base AN value for 0311
0023	0021 | 0025	15.00	2.00	2.00	R 79.79	R 79.79	Time calculation for 0023
0035		1.00	2.00	2.00	R 79.79	R 79.79	Minimum units rule
TOTALS:			7.00	7.00		279.27	

2.4.9.1.f Anaesthetist (GP) claim with a duration of <= 60 minutes where other modifiers must be applied:

Duration: 15 minutes and 2006 NHRPL Anaesthetic RCF = R 39.895

Proc. code	Modifier(s)	Qty	Original Anaesthetic Units	Calculated Anaesthetic Units	Calculated Amount	Rounded off Amount	Calculation
1101		1.00	4.00	4.00	R 159.58	R 159.58	Base AN value for 1101
0023	0021 | 0025	15.00	2.00	2.00	R 79.79	R 79.79	Time calculation for 0023
0035		1.00	1.00	1.00	R 39.895	R 39.90	Minimum units rule
TOTALS:			7.00	7.00		R 279.27	

2.4.9.1.g Anaesthesiologist (Specialist) claim with a duration of > 60 minutes where other modifiers must be applied:

Duration: 75 minutes and 2006 NHRPL Anaesthetic RCF = R 39.895

Proc. code	Modifier(s)	Qty	Original Anaesthetic Units	Calculated Anaesthetic Units	Calculated Amount	Rounded off Amount	Calculation
1276		1.00	13.00	13.00	R 518.635	R 518.64	Base AN value for 1276
0032		1.00	0.00	0.00	R 0.00	R 0.00	Patient in prone position
0039		75.00	4.00	4.00	R 159.58	R 159.58	Control of blood pressure
0023	0021 | 0025	75.00	11.00	11.00	R 438.845	R 438.85	Time calculation for 0023
0018		1.00	5.50	5.50	R219.4225	R 219.42	BMI > 35
TOTALS:			33.50	33.50		R 1336.49	

2.4.9.1.h Anaesthetist (GP) claim with a duration of > 60 minutes where other modifiers must be applied:

Duration: 75 minutes and 2006 NHRPL Anaesthetic RCF = R 39.895

Proc. code	Modifier(s)	Qty	Original Anaesthetic Units	Calculated Anaesthetic Units	Calculated Amount	Rounded off Amount	Calculation
1149	0036	1.00	11.00	8.80	R 351.076	R 351.08	Base AN value for 1149 MINUS mod 0036 [20%]
0032	0036	1.00	0.00	0.00	R 0.00	R 0.00	Patient in prone position MINUS mod 0036 [20%]
0039	0036	75.00	4.00	3.20	R 127.664	R 127.66	Control of blood pressure MINUS mod 0036 [20%]
0023	0036 | 0021| 0025	75.00	11.00	8.80	R 351.076	R 351.08	Time calculation for 0023 [11.00 AN units] MINUS mod 0036 [20%]
0018	0036	1.00	5.50	4.40	R 175.538	R 175.54	BMI > 35 MINUS mod 0036 [20%]
TOTALS:			31.50	25.20		R 1005.36	

2.4.9.2 Examples of Unit Calculations for Medical Practitioner modifier 0036

	Procedure code	: Contains Basic Anaesthetic Units	
+	Modifier rules and information	: Add, Reduce, Informational	: Indicated as Proc
+	Anaesthetic Time Modifier 0023	: Contains Time Units	: Indicated as N/A
+	Modifier rules and information	: Add, Reduce, Informational	: Indicated as 0023

		A.	B.	C.	D.	E.	F.	G.	H.
	Time in minutes (15 min periods or part thereof)	**Basic Procedure Anaesth. Units**	***Time units <= 1 hour***	***Time units > 1 hour***	**Total Time units (B+C) (0023)**	**Subtotal (A+D)**	***Minimum of 7.00 Total AN. Units (0035)***	**Total (E+F)**	**GPs > 1 hour: 80% with minimum of 11.00 units (0036)**
1/4 hour	15	3.00	*2.00*		2.00	5.00	*2.00*	7.00	
1/2 hour	30	3.00	*4.00*		4.00	7.00		7.00	
3/4 hour	45	3.00	*6.00*		6.00	9.00		9.00	
1 hour	60	3.00	*8.00*		8.00	11.00		11.00	
1 1/4 hour	75	3.00	*8.00*	*3.00*	11.00	14.00			11.20
1 1/2 hour	90	3.00	*8.00*	*6.00*	14.00	17.00			13.60
1 3/4 hour	105	3.00	*8.00*	*9.00*	17.00	20.00			16.00
2 hours	120	3.00	*8.00*	*12.00*	20.00	23.00			18.40

		A.	B.	C.	D.	E.	F.	G.	H.
	Time in minutes (15 min periods or part thereof)	**Basic Procedure Anaesth. Units**	***Time units <= 1 hour***	***Time units > 1 hour***	**Total Time units (B+C) (0023)**	**Subtotal (A+D)**	***Minimum of 7.00 Total AN. Units (0035)***	**Total (E+F)**	**GPs > 1 hour: 80% with minimum of 11.00 units (0036)**
1/4 hour	15	4.00	*2.00*		2.00	6.00	*1.00*	7.00	
1/2 hour	30	4.00	*4.00*		4.00	8.00		8.00	
3/4 hour	45	4.00	*6.00*		6.00	10.00		10.00	
1 hour	60	4.00	*8.00*		8.00	12.00		12.00	
1 1/4 hour	75	4.00	*8.00*	*3.00*	11.00	15.00			12.00
1 1/2 hour	90	4.00	*8.00*	*6.00*	14.00	18.00			14.40
1 3/4 hour	105	4.00	*8.00*	*9.00*	17.00	21.00			16.80
2 hours	120	4.00	*8.00*	*12.00*	20.00	24.00			19.20

		A.	B.	C.	D.	E.	F.	G.	H.
	Time in minutes (15 min periods or part thereof)	**Basic Procedure Anaesth. Units**	***Time units <= 1 hour***	***Time units > 1 hour***	**Total Time units (B+C) (0023)**	**Subtotal (A+D)**	***Minimum of 7.00 Total AN. Units (0035)***	**Total (E+F)**	**GPs > 1 hour: 80% with minimum of 11.00 units (0036)**
1/4 hour	15	5.00	*2.00*		2.00	7.00		7.00	
1/2 hour	30	5.00	*4.00*		4.00	9.00		9.00	
3/4 hour	45	5.00	*6.00*		6.00	11.00		11.00	
1 hour	60	5.00	*8.00*		8.00	13.00		13.00	
1 1/4 hour	75	5.00	*8.00*	*3.00*	11.00	16.00			12.80
1 1/2 hour	90	5.00	*8.00*	*6.00*	14.00	19.00			15.20
1 3/4 hour	105	5.00	*8.00*	*9.00*	17.00	22.00			17.60
2 hours	120	5.00	*8.00*	*12.00*	20.00	25.00			20.00

		A.	B.	C.	D.	E.	F.	G.	H.
	Time in minutes (15 min periods or part thereof)	**Basic Procedure Anaesth. Units**	***Time units <= 1 hour***	***Time units > 1 hour***	**Total Time units (B+C) (0023)**	**Subtotal (A+D)**	***Minimum of 7.00 Total AN. Units (0035)***	**Total (E+F)**	**GPs > 1 hour: 80% with minimum of 11.00 units (0036)**
1/4 hour	15	6.00	*2.00*		2.00	8.00		8.00	
1/2 hour	30	6.00	*4.00*		4.00	10.00		10.00	
3/4 hour	45	6.00	*6.00*		6.00	12.00		12.00	
1 hour	60	6.00	*8.00*		8.00	14.00		14.00	
1 1/4 hour	75	6.00	*8.00*	*3.00*	11.00	17.00			13.60
1 1/2 hour	90	6.00	*8.00*	*6.00*	14.00	20.00			16.00
1 3/4 hour	105	6.00	*8.00*	*9.00*	17.00	23.00			18.40
2 hours	120	6.00	*8.00*	*12.00*	20.00	26.00			20.80

		A.	B.	C.	D.	E.	F.	G.	H.
	Time in minutes (15 min periods or part thereof)	**Basic Procedure Anaesth. Units**	***Time units <= 1 hour***	***Time units > 1 hour***	**Total Time units (B+C) (0023)**	**Subtotal (A+D)**	***Minimum of 7.00 Total AN. Units (0035)***	**Total (E+F)**	**GPs > 1 hour: 80% with minimum of 11.00 units (0036)**
1/4 hour	15	7.00	*2.00*		2.00	9.00		9.00	
1/2 hour	30	7.00	*4.00*		4.00	11.00		11.00	
3/4 hour	45	7.00	*6.00*		6.00	13.00		13.00	
1 hour	60	7.00	*8.00*		8.00	15.00		15.00	
1 1/4 hour	75	7.00	*8.00*	*3.00*	11.00	18.00			14.40
1 1/2 hour	90	7.00	*8.00*	*6.00*	14.00	21.00			16.80
1 3/4 hour	105	7.00	*8.00*	*9.00*	17.00	24.00			19.20
2 hours	120	7.00	*8.00*	*12.00*	20.00	27.00			21.60

		A.	B.	C.	D.	E.	F.	G.	H.
	Time in minutes (15 min periods or part thereof)	**Basic Procedure Anaesth. Units**	***Time units <= 1 hour***	***Time units > 1 hour***	**Total Time units (B+C) (0023)**	**Subtotal (A+D)**	***Minimum of 7.00 Total AN. Units (0035)***	**Total (E+F)**	**GPs > 1 hour: 80% with minimum of 11.00 units (0036)**
1/4 hour	15	8.00	*2.00*		2.00	10.00		10.00	
1/2 hour	30	8.00	*4.00*		4.00	12.00		12.00	
3/4 hour	45	8.00	*6.00*		6.00	14.00		14.00	
1 hour	60	8.00	*8.00*		8.00	16.00		16.00	
1 1/4 hour	75	8.00	*8.00*	*3.00*	11.00	19.00			15.20
1 1/2 hour	90	8.00	*8.00*	*6.00*	14.00	22.00			17.60
1 3/4 hour	105	8.00	*8.00*	*9.00*	17.00	25.00			20.00
2 hours	120	8.00	*8.00*	*12.00*	20.00	28.00			22.40

		A.	B.	C.	D.	E.	F.	G.	H.
	Time in minutes (15 min periods or part thereof)	**Basic Procedure Anaesth. Units**	***Time units <= 1 hour***	***Time units > 1 hour***	**Total Time units (B+C) (0023)**	**Subtotal (A+D)**	***Minimum of 7.00 Total AN. Units (0035)***	**Total (E+F)**	**GPs > 1 hour: 80% with minimum of 11.00 units (0036)**
1/4 hour	15	9.00	*2.00*		2.00	11.00		11.00	
1/2 hour	30	9.00	*4.00*		4.00	13.00		13.00	
3/4 hour	45	9.00	*6.00*		6.00	15.00		15.00	
1 hour	60	9.00	*8.00*		8.00	17.00		17.00	
1 1/4 hour	75	9.00	*8.00*	*3.00*	11.00	20.00			16.00
1 1/2 hour	90	9.00	*8.00*	*6.00*	14.00	23.00			18.40
1 3/4 hour	105	9.00	*8.00*	*9.00*	17.00	26.00			20.80
2 hours	120	9.00	*8.00*	*12.00*	20.00	29.00			23.20

		A.	B.	C.	D.	E.	F.	G.	H.
	Time in minutes (15 min periods or part thereof)	**Basic Procedure Anaesth. Units**	***Time units <= 1 hour***	***Time units > 1 hour***	**Total Time units (B+C) (0023)**	**Subtotal (A+D)**	***Minimum of 7.00 Total AN. Units (0035)***	**Total (E+F)**	**GPs > 1 hour: 80% with minimum of 11.00 units (0036)**
1/4 hour	15	10.00	*2.00*		2.00	12.00		12.00	
1/2 hour	30	10.00	*4.00*		4.00	14.00		14.00	
3/4 hour	45	10.00	*6.00*		6.00	16.00		16.00	
1 hour	60	10.00	*8.00*		8.00	18.00		18.00	
1 1/4 hour	75	10.00	*8.00*	*3.00*	11.00	21.00			16.80
1 1/2 hour	90	10.00	*8.00*	*6.00*	14.00	24.00			19.20
1 3/4 hour	105	10.00	*8.00*	*9.00*	17.00	27.00			21.60
2 hours	120	10.00	*8.00*	*12.00*	20.00	30.00			24.00

3. Appendix A: Summary of Indicator Tables

The following is a summary of all the Tables referred to in this document:-

- Table 1: Council Type Indicators

01	HPCSA	Health Professions Council of South Africa
02	AHPCSA	Allied Health Professions Council of South Africa
03	SACSSP	SA Council for Social Services Professions
04	SADTC	SA Dental Technician Council
05	SANC	SA Nursing Council
06	SAPC	SA Pharmacy Council

- Table 4: Place of Service Code Indicators

21	Inpatient Hospital	52	Acute psychiatric facility
51	Inpatient psychiatric hospital	53	Out of hospital Mental Health Clinic
61	Inpatient rehabilitation facility	55	Substance Rehabilitation
11	Consulting room	56	Halfway House
12	Home	62	Outpatient rehabilitation
22	Hospital Outpatient facility	65	Dialysis Centre
23	Casualty	66	Radiotherapy Treatment Centre
24	Day Clinic / Hospital	68	Chemotherapy Treatment Centre
25	Birthing Centre	70	Oncology Centre
26	Military treatment facility	71	State or Local Public Health Clinic
31	Step-down	72	Rural Health Clinic
32	Nursing Home	81	Private Laboratory
33	Chronic Psychiatric Facility	84	Independent Pharmacy
34	Hospice	85	Hospital Pharmacy
41	Ambulance – Land	99	Other unlisted facility
42	Ambulance – Air or Water		

- Table 5: Procedural Coding Standard Indicators

01	NHRPL
02	CPT / CCSA
03	CDT
04	Proprietary Code Set
05	SAOA
06	SAOPA
07	SAMA (South African Medical Association)

- Table 6: Item Type Indicators

01	Product Code only
02	Tariff / Procedure Code only
03	Tariff / Procedure Code and Modifier Code
04	Tariff / Procedure Code and Product Code
05	Modifier Code only
06	Dental Tariff / Procedure Code and Dental Laboratory Tariff / Procedure Code

Item Type Indicators 03, 05 and 06 will never be used on Optometry Claims.

- Table 7: Basis of Days of Therapy Indicators

00	Not Specified
01	Implicit Usage
02	Dispensers Estimate
03	Doctors Directions

- Table 8: Mixture Code Indicators

00	Not an ingredient of a mixture
01	Item is an ingredient of a mixture
02	Item is an ingredient of a mixture and includes mixture fee

- Table 9: Pricing Tariff Indicators

01	NHRPL Rate
02	Private Tariff Rate
03	Medical Scheme Rate
04	Contractual Rate
05	Manufacturer's Price
06	COID
07	Ethical Tariff Rates
08	Unknown
09	Single Exit Price (SEP)
10	Nett Acquisition Price (NAP)

- Table 10: Transaction Type Indicators and Descriptions

Transaction Type Indicator	Transaction Type Description	Additional Information
PHISC01	Claim	Used for both paper and electronic Claims.
PHISC02	Amended Claim	Used for both paper and electronic Claims.
PHISC03	Credit Note	Used for both paper and electronic Credit Notes.
PHISC04	COID Claim	Used to show that the Claim needs to be submitted to the Compensation Commissioner and not the Medical Scheme for reimbursement. Not used for electronic Claims.
PHISC05	COID Amended Claim	Used to show that the Amended Claim needs to be submitted to the Compensation Commissioner and not the Medical Scheme for reimbursement. Not used for electronic Claims.
PHISC06	COID Credit Note	Used to show that the Credit Note needs to be submitted to the Compensation Commissioner and not the Medical Scheme for reimbursement. Not used for electronic Credit Notes.
PHISC07	Response Message	Used for electronic Response messages.

- Table 11: Registration Types

01	Council for Medical Schemes (CMS)
02	Private Healthcare Industry Standards Committee (PHISC)

- Table 12: Abbreviations for Tooth Surfaces

B	Buccal (Vestibular)
D	Distal
L	Labial (Vestibular)
L	Lingual
M	Mesial
O	Occlusal (or incisal)
P	Palatal

- Table 13: Benefit Type Indicator

001	Acute
002	Appliances
003	Chemotherapy
004	Chronic
005	Dialysis
006	Ex Gratia
007	General Practitioners
008	HIV
009	Hospital
010	Life Compromising
011	Life Sustaining
012	Life Threatening
013	Major Medical
014	Oncology
015	Organ Transplant
016	Over-The-Counter (OTC)
017	Prosthesis
018	Prescribed Minimum Benefits (PMB)
019	Specialist
020	Trauma

- Table14: Deduction Type Indicators

01	Levy
02	Co-payment

- Table 15: Product Coding Standard Indicators

01	NAPPI
02	SAOA
03	SAOPA
04	Proprietary Code Set

- Table 16: Response Tracking Status Indicators

Response Tracking Status Indicators	Response Tracking Status Description
Received	As per the definition of a Confirmation of Receipt Response message: The message was parsed, complies with the PHISC XML schema and persisted to a non-volatile storage device and has been accepted for further processing.
Switched	The message was 'Received' and passed onto another trading party for further processing.
Partly Processed	The message was partly processed and the Response message contains the result of this processing, but that further Response messages should be expected from this or other trading parties.
Fully Processed	This message was fully processed and the Response message contains the final result of processing. The Processing Intermediary of this result assures that no further Response messages should be expected on this message from any other parties in the trading community.
Failed	The recipient of the message was unable to validate or process the message for reasons indicated in the Response message.

- Table 17: Response Process Status Indicators

Response Process Status Indicators	Response Process Status Description
Received	The item was parsed, complies with the PHISC XML schema and persisted to a non-volatile storage device.
Valid	The item conforms to the mandatory information and data integrity requirements of the recipient.
Invalid	The item is missing mandatory information or contains invalid information according to the mandatory information and data integrity requirements of the recipient.
Rejected	The item has been successfully processed BUT the Claim item / Credit Note item has been rejected and no payment for this item will be made.
Adjusted	The item has been successfully processed BUT although payment has been adjudicated for the item, payment will be less than the Claimed Amount.
Pended	The item has been Received but is still being processed. No further information is available at the time of generating the Response message,
Referred	The item has been Received but has been referred to another party for processing. No further information is available at the time of generating the Response message,
Approved	The item has been successfully processed AND the amount claimed has been approved for payment as requested.
Paid	The item has been successfully processed AND paid. This status is expected to be restricted for use in the Electronic Remittance Advice.

- Table 18: Response Reason Code Type Indicators

Response Reason Code Value	Response Reason Code Description
Rejection	The Claim / Credit Note will not be processed further as a consequence of the reason provided.
Warning	The Reason Code and description has a material impact on the processing and payment of the Claim. There is likely to be a difference between the Billing Practice / Supplier's expectation of payment and the final adjudication.
Information	The Reason Code and description provides additional information which the Billing Practice / Supplier should take cognisance of.
Unknown	In instances where a processing intermediary is not able to determine the Response Reason Code Type Indicator, 'Unknown' should be used.

- Table 19: Response Reason Code Source Indicators

TRUE	PHISC as source
FALSE	Other party as source

- Table 20: Patient Gender Indicators

F	Female
M	Male
U	Unknown

- Table 21: Non-provider Trading Party Identifiers

PHISC Identifier	Non-Provider Trading Party's Name
EMD001	eMD
SWI001	DH Switch
GDX001	GoodX
HBR001	HealthBridge
LAN001	Lancet

- Table 22: Reason Code Type Qualifiers

Qualifier	Description
01	Invalid / Missing Policy Data
02	Invalid / Missing Practice or Provider Data
03	Invalid / Missing Coding Data
04	Invalid Message Integrity
05	Funder Rules
06	Inappropriate Clinical Application
07	Technical Error

- Table 23: Advisory Action Indicators

Qualifier	Description
01	No Action
02	Verify and / or correct data: Reversal
03	Verify and / or correct data: Resend
04	Member Liable
05	Destination not available, try again later
06	Refer to reason code for action

4. Appendix B: Financial Element Examples

The Claim and Claim Response messages make provision for a number of financial fields where the usage of these elements is illustrated in the following examples.

4.1 Step 1: Billing Practice / Supplier submits claim

A service provider consults with a patient and submits a Claim to the medical scheme as set out below:

Billing Event		Item 1 (Clinical Encounter)	
	Submitted		**Submitted**
Gross Amount	**125.00**	**Gross Amount**	**125.00**
		Pricing Tariff Indicator	*NHRPL*
Total Discounts	**15.00**	*Base Cost* [1]	95.00
Billing Event Discount	0	*Mark-Up* [3]	30.00
Clinical Encounter Discounts	15.00	*Professional Fee* [2]	0
Total Collections	**25.00**	Line Item Discount [4]	**15.00**
Billing Event Collections	0		
Clinical Encounter Collections	25.00	Line Item Collections [5]	**25.00**
Claim Amount	**85.00**	Claim Amount	**85.00**
VAT Amount	**11.90**		

Legend:

1. The Base Cost of the visit is based on NHRPL and is R 95.00
2. No products were dispensed or used during treatment; therefore the Professional Fee does not apply to this single line claim.
3. The service provider deems a mark-up of R 30.00 above the NHRPL rate should be charged based on his unique experience, skill set and demand for his specific services in the region. This gives rise to a Submitted Gross Amount of R 125.00.
4. However, the service provider has a preferred provider arrangement with this patient's medical scheme and the agreement includes a 12% discount on the Gross Amount to be applied for all medical scheme members. The Submitted Discount on the claim is therefore R 15.00 for this visit.
5. The service provider is also aware that certain levies apply and the patient is concerned that he could be running out of benefits. As a consequence the service provider elects to collect R 25.00 from the patient at the point-of-care and submit the remainder of the claim, R 85.00, to the medical scheme for re-imbursement.

4.2 Step 2: Processing Intermediary evaluate claim

Billing Event			Item 1 (Clinical Encounter)		
	Submitted	**Calculated**		**Submitted**	**Calculated**
Gross Amount	**125.00**	**127.50**	**Gross Amount**	**125.00**	**127.50**
			Pricing Tariff Indicator	*NHRPL*	
Total Discounts	**15.00**	**19.13**	*Base Cost* [1]	95.00	85.00
Billing Event Discount	0	0	*Mark-Up* [2]	30.00	42.50
Clinical Encounter Discount	15.00	19.13	*Professional Fee*	0	0
Total Collections	**25.00**		**Line item Discount** [3]	**15.00**	**19.13**
Billing Event Collections	0				
Clinical Encounter Collections	25.00		**Line item Collections**	**25.00**	
Total Patient Liable Deductions		**15.00**	**Total Patient Liable Deductions**		**15.00**
Billing Event Deductions		0	Deduction on Levy[4]		10.00
Clinical Encounter Deductions: Levy		10.00	Deduction on Co-pay[5]		5.00
Clinical Encounter Deductions: Co-pay		5.00			
Claim Amount	**85.00**	93.37	**Claim Amount**	**85.00**	93.37
VAT Amount	11.90	13.07			

Legend:

1. As part of the agreed arrangement with the billing practice, the correct Pricing Tariff Indicator of 'NHRPL' was used, but the correct NHRPL price for the item is in fact R 85.00 (not R 95.00) as per the latest price file.
2. The contractual arrangement also makes provision for an additional professional fee of 50% above NHRPL which in this case puts the Calculated Professional Fee at R 42.50 and the Calculated Gross Amount at R 127.50.
3. The medical scheme determines that the service provider is still using the discount ratio from the previous year's contract which is now higher, at 15% of Gross Amount; a Calculated Discount of R 19.13.
4. The medical scheme also determines that the medical scheme rules require patients to pay a R 10.00 levy for each of these type of consultations.
5. The medical scheme then assesses the claim against the patient's benefits and limits and determines that there will be a shortfall of R 5.00 which manifests as a patient co-payment.

4.3 Step 3: Approval of claim

Billing Event				Item 1 (Clinical Encounter)			
	Submitted	**Calculated**	**Approved**		**Submitted**	**Calculated**	**Approved**
Gross Amount	**125.00**	**127.50**	**125.00**	**Gross Amount**[1]	**125.00**	**127.50**	**125.00**
				Pricing Tariff Indicator	*NHRPL*		
Total Discounts	**15.00**	**19.13**	**19.13**	*Base Cost*	95.00	85.00	85.00
Billing Event Discount	0	0	0	*Mark-Up*	30.00	42.50	42.50
Clinical Encounter Discount	15.00	19.13	19.13	*Prof. Fee*	0	0	
Total Collections	**25.00**			**Line item Discount** [2]	**15.00**	**19.13**	**19.13**
Billing Event Collections	0						
Clinical Encounter Collections	25.00			**Line item Collections**	**25.00**		
Total Patient Liable Deductions		**15.00**	**15.00**	**Total Patient Liable Deductions**[3]		**15.00**	**15.00**
Billing Event Deductions		0		Deduction on Levy		10.00	
Clinical Encounter Deductions: Levy		10.00		Deduction on Co-pay		5.00	
Clinical Encounter Deductions: Co-pay		5.00					
Claim Amount	**85.00**	**93.37**	90.87	Claim Amount[4]	**85.00**	93.37	90.87
VAT Amount	11.90	13.07	12.72				
Due by Payor to Member			**0.00**				
Due by Billing Practice to Member			**10.00**				

Legend:

1. Since the billing practice only charged a Submitted Gross of R 125.00, the Approved Gross is equal to the Submitted Gross and not the Calculated Gross.
2. The medical scheme also applies the current discount arrangement and sets the Approved Discount equal to R 19.13.
3. The levy and co-payment results in a Calculated Total Patient Liable Deduction of R 15.00 which is taken as the Approved Total Patient Liable Deduction and not the Collections determined by the billing practice.
4. Ultimately the medical scheme determines that an amount of R 90.87 is the Approved Claim Amount and subject to the final payment run will be paid to the billing practice.
5. Since the billing practice collected R 10.00 more from the member than the Approved Total Patient Liable Deductions amount, the medical scheme indicates in the Response message that the Due by Billing Practice to Member is R 10.00.

Note:
If the contractual arrangement allowed for the lesser of Submitted and Calculated Base Cost, Mark-Up and Professional Fee to be used then the following values would have been approved:

- Approved Base Cost = R 85.00
- Approved Mark-Up = R 30.00
- Approved Gross = R 115.00
- Approved Discount = R 19.13
- Approved Patient Deductions = R 15.00
- Approved Claimed Amount = R 80.87

4.4 Reimbursement Scenario 1

The Payor must recover the excessive out-of-pocket expense from the billing practice's Amount Due (Approved Claimed Amount) and settles directly with the member.

- Set Approved Total Patient Liable Deductions = Collections = R 25.00
- Therefore: Due By Payor To Member = Approved Total Patient Liable Deductions **minus** Calculated Total Patient Liable Deductions = R 25.000 **minus** R 15.00 = R10.00
- And: Due By Provider To Member = Collections **minus** Approved Total Patient Liable Deductions = R25.00 **minus** R 25.00 = R 0.00

The net effect of this scenario is that the Processing Intermediary reduces the Approved Claim Amount by the Calculated Deductions as well as the excess amount collected. The Processing Intermediary then reimburses the member directly with the excess amount recovered from the billing practice.

Illustrated as follows:

	Submitted	Calculated	Approved	
Gross Amount	**125.00**	**125.00**	**125.00**	
Pricing Tariff Indicator	*NHRPL*	*NHRPL*		
Base Cost	95.00	95.00		
Mark-Up	0.00	0.00		
Professional Fee	30.00	30.00		
Less Discount	**0.00**	**0.00**	**0.00**	
Less Collected Amounts	**25.00**			
Total Patient Liable Deductions		**15.00**	**25.00**	
Deduction: Levy		0.00		
Deduction: Co-Pay		15.00		
Claim Amount	**100.00**	**110.00**	**100.00**	
			Due by Payor to Member	**10.00**
			Due by Billing Practice to Member	**0.00**

4.5 Reimbursement Scenario 2

The Payor settles with the Billing Practice Provider as per the Calculated Deductions, but requires the Billing Practice to settle excess out-of-pocket collections with the Member.

- Set Approved Total Patient Liable Deductions = Calculated Total Patient Liable Deductions = R 15.00
- Therefore: Due By Payor To Member = Approved Total Patient Liable Deductions **minus** Calculated Total Patient Liable Deductions = R 15.00 **minus** R 15.00 = R 0.00
- And: Due By Provider To Member = Collections **minus** Approved Total Patient Liable Deductions = R 25.00 **minus** R 15.00 = R 10.00

The effect of this is that the Processing Intermediary only reduces the Approved Claim Amount by the deductions that should have been collected irrespective of what was actually collected, but requires the billing practice to settle the R 10.00 excessive out-of-pocket expense directly with the member,

Illustrated as follows:

	Submitted	Calculated	Approved	
Gross Amount	**125.00**	**125.00**	**125.00**	
Pricing Tariff Indicator	*NHRPL*	*NHRPL*		
Base Cost	95.00	95.00		
Mark-Up	0.00	0.00		
Professional Fee	30.00	30.00		
Less Discount	**0.00**	**0.00**	**0.00**	
Less Collected Amounts	**25.00**			
Total Patient Liable Deductions		**15.00**	**15.00**	
Deduction: Levy		0.00		
Deduction: Co-Pay		15.00		
Claim Amount	**100.00**	**110.00**	**110.00**	
			Due by Payor to Member	**0.00**
			Due by Billing Practice to Member	**10.00**

4.6 Patient Claim Reimbursement Scenario 3

These elements can also be used in a patient claim scenario in which the billing practice collects the full Submitted Claim Amount upfront, but still submits the Claim to the Processing Intermediary as an added service to the member such that the Payor can reimburse the member directly. In such a case the Claim must be sent with the 'Do Pay Member' element set to 'True'.

Illustrated as follows:

	Submitted	Calculated	Approved	
Gross Amount	**125.00**	**130.00**	**125.00**	
Pricing Tariff Indicator	*NHRPL*	*NHRPL*		
Base Cost	95.00	85.00		
Mark-Up	0.00	0.00		
Professional Fee	30.00	45.00		
Less Discount	**10.00**	**19.50**	**19.50**	
Less Collected Amounts	**115.00**			
Total Patient Liable Deductions		**15.00**	**105.50**	
Deduction: Levy		0.00		
Deduction: Co-Pay		15.00		
Claim Amount	**0.00**	**95.50**	**0.00**	
			Due by Payor to Member	**90.50**
			Due by Billing Practice to Member	**9.50**

This is based on:

- Setting Approved Total Patient Liable Deductions = Approved Gross **minus** Approved Discount = R 105.50
- Therefore: Due By Payor To Member = Approved Total Patient Liable Deductions **minus** Calculated Total Patient Liable Deductions = R 105.50 **minus** R 15.00 = R 90.50
- And: Due By Provider To Member = Collections **minus** Approved Total Patient Liable Deductions = R 115.00 **minus** R 105.50 = R 9.50

5. Appendix C: PHISC List of CMS Unregistered / Exempt Medical Schemes

The following is a list of all the Medical Schemes excluded / exempt from registration with the Council for Medical Schemes (CMS). Please refer to section 2.3.3 Medical Scheme Information for more information on its use.

PHISC Reference No.	Scheme Name
PHISC001	Bargaining Council for Fabric Knitting (Northern Areas KwaZulu Natal)
PHISC002	Bargaining Council for Hairdressing and Cosmetology (KwaZulu Natal)
PHISC003	Bargaining Council for Hairdressing and Cosmetology (Semi-National)
PHISC004	Bargaining Council for the Bag Manufacturing Industry (Paarl)
PHISC005	Bargaining Council for the Building Industry (Bloemfontein)
PHISC006	Bargaining Council for the Building Industry (Gauteng)
PHISC007	Bargaining Council for the Building Industry (Kimberley)
PHISC008	Bargaining Council for the Canvas Goods Industry (Witwatersrand and Pretoria)
PHISC009	Bargaining Council for the Canvas Rope Working Industry (Cape)
PHISC010	Bargaining Council for the Central Statistical Service at Department Level
PHISC011	Bargaining Council for the Clothing Industry (Northern Natal)
PHISC012	Bargaining Council for the Contract Cleaning Industry (KwaZulu Natal)
PHISC013	Bargaining Council for the Department of Arts, Culture, Science and Technology
PHISC014	Bargaining Council for the Department of Constitutional Development
PHISC015	Bargaining Council for the Department of Correctional Services
PHISC016	Bargaining Council for the Department of Defence
PHISC017	Bargaining Council for the Department of Environmental Affairs and Tourism
PHISC018	Bargaining Council for the Department of Foreign Affairs
PHISC019	Bargaining Council for the Department of Housing
PHISC020	Bargaining Council for the Department of Justice
PHISC021	Bargaining Council for the Department of Labour
PHISC022	Bargaining Council for the Department of Minerals and Energy
PHISC023	Bargaining Council for the Department of Public Works
PHISC024	Bargaining Council for the Department of State Expenditure
PHISC025	Bargaining Council for the Department of Water Affairs and Forestry
PHISC026	Bargaining Council for the Department of Welfare
PHISC027	Bargaining Council for the Diamond Cutting Industry of South Africa (National)
PHISC028	Bargaining Council for the Dissolving Pulp Manufacturing Industry
PHISC029	Bargaining Council for the Entertainment Industry of South Africa (National)
PHISC030	Bargaining Council for the Fishing Industry
PHISC031	Bargaining Council for the Furniture Manufacturing Industry (Free State)
PHISC032	Bargaining Council for the Furniture Manufacturing Industry (KwaZulu Natal)
PHISC033	Bargaining Council for the Furniture Manufacturing Industry (South Western Districts)
PHISC034	Bargaining Council for the Furniture Manufacturing Industry (Western Cape)
PHISC035	Bargaining Council for the Furniture, Bedding and Upholstery Industry (Greater Northern Region (FBUIBC GNR)
PHISC036	Bargaining Council for the Hairdressing and Cosmetology Trade (Pretoria)
PHISC037	Bargaining Council for the Hairdressing Trade (Cape Peninsula)
PHISC038	Bargaining Council for the Hosiery Industry (KwaZulu Natal)
PHISC039	Bargaining Council for the Jewellery and Precious Metal Industry (Cape)
PHISC040	Bargaining Council for the Laundry, Dry Cleaning and Dyeing Industry (Cape)
PHISC041	Bargaining Council for the Laundry, Dry Cleaning and Dyeing Trade (Gauteng)
PHISC042	Bargaining Council for the Laundry, Dry Cleaning and Dyeing Trade (KwaZulu Natal)
PHISC043	Bargaining Council for the Liquor, Catering and Accommodation Trades (South Coast, KwaZulu Natal)
PHISC044	Bargaining Council for the Meat Trade (Gauteng)

PHISC Reference No.	Scheme Name
PHISC045	Bargaining Council for the Meat Trade (Magisterial District of East London)
PHISC046	Bargaining Council for the Millinery Industry (Cape)
PHISC047	Bargaining Council for the Millinery Industry (Northern Areas)
PHISC048	Bargaining Council for the Mpumalanga Provincial Administration
PHISC049	Bargaining Council for the New Tyre Manufacturing
PHISC050	Bargaining Council for the Non-European Passengers Transportation TradeTRADE
PHISC051	Bargaining Council for the Provincial Administration (Northern Transvaal)
PHISC052	Bargaining Council for the Public Service Commission
PHISC053	Bargaining Council for the Restaurant, Catering and Allied Trades (Witwatersrand)
PHISC054	Bargaining Council for the South African Police Service
PHISC055	Bargaining Council for the Sugar Manufacturing and Refining Industry
PHISC056	Bargaining Council for the Sweet Making Industry (Cape)
PHISC057	Bargaining Council for the Tearoom, Restaurant and Catering Trade (Pretoria)
PHISC058	Bargaining Council for the Wool and Mohair Industry
PHISC059	Bargaining Council for the Worsted Textile Manufacturing Industry (Cape)
PHISC060	Building and Construction Industry Medical Aid - BIMAF (East London)
PHISC061	Building and Construction Industry Medical Aid - BIMAF (North & West Boland)
PHISC062	Building and Construction Industry Medical Aid - BIMAF (Southern & Eastern Cape)
PHISC063	Clothing Industry Health Care Fund - CIHCF (Free State and Northern Cape)
PHISC064	Clothing Industry Health Care Fund - CIHCF (Kwa-Zulu Natal)
PHISC065	Clothing Industry Health Care Fund - CIHCF (Northern Areas)
PHISC066	Clothing Industry Health Care Fund - CIHCF (Western Cape)
PHISC067	General Public Service Sectoral Bargaining Council
PHISC068	Kimberley Commercial Distributive Trade Bargaining Council
PHISC069	Metal and Engineering Industries Bargaining Council (National)
PHISC070	Motor Ferry Industry Bargaining Council of South Africa
PHISC071	Motor Industry Bargaining Council - MIBCO (National)
PHISC072	Public Health and Welfare Sectoral Bargaining Council
PHISC073	Public Service Bargaining Council for the Department of Health
PHISC074	Public Service Bargaining Council for the Department of Transport
PHISC075	Public Service Bargaining Council of the Provincial Administration (Western)
PHISC076	Public Service Co-ordinating Bargaining Council
PHISC077	Safety and Security Sectoral Bargaining Council
PHISC078	Transnet Bargaining Council (National)

6. Appendix D: Contact Details

The following is a list of all Professional Associations, their contact number(s) and which medical practice type is represented by them:-

	Professional Association	Contact Number	Service Provider Field of Study
1.	Biokinetics Association	082 – 870 6115	Biokinetics
2.	Chiropractic Association of South Africa	058 – 303 4571	Chiropractors Homeopathy Masseurs
3.	Democratic Nursing Association of SA	012 – 343 2315	Registered Nurses
4.	Dieticians of South Africa Association	013 – 656 6466	Dieticians
5.	Hospital Association of SA (HASA)	011 – 478 0156	Private Hospitals
6.	Occupational Therapy Association of SA	012 – 365 1327	Occupational Therapy
7.	Optometric Association of SA	011 – 805 4517	Optical Dispensing Optometry Supplementary Optometry Supplementary Optical Dispensing
8.	Orthotists Association	011 – 442 6080	Orthotists Prosthetists
9.	Orthoptists Association	011 – 442 6080	Orthoptists
10.	Pharmaceutical Association of SA	012 – 301 0820	Pharmacies Pharmacotherapy
11.	Physiotherapy Society of SA	012 – 485 1467	Physiotherapy
12.	Psychological Association of SA	011 – 486 3322	Psychology Psychometry Registered Councellors
13.	Society of Medical Laboratory Technicians	011 – 852 2458	Medical Science Medical Technology
14.	South African Association of Audiologists	016 – 933 6386	Audiology
15.	South African Association of Social Workers of SA	011 – 887 1968	Social Workers
16.	South African Dental Association (SADA)	011 – 484 5288	General Dental Practice Maxillo-Facial and Oral Surgery Orthodontics
17.	South African Hearing Association (SASLHA)	011 – 653 2114	Hearing Aid Acousticians

	Professional Association	Contact Number	Service Provider Field of Study
18.	South African Medical Association (SAMA)	012 – 481 2000	Anaesthesiology Cardiology Cardio-Thoracic Surgery Clinical Haematology Dermatology Diagnostic Radiology Emergency Medicine Endocrinology Gastroenterology General Medical Practices Internal Medicine Medical Oncology Nephrology Neurology Neurosurgery Nuclear Medicine Nuclear Medicine Obstetrics and Gynaecology Oncology Ophthalmology Orthopaedics Paediatrics Paediatric Cardiology Pathology Physical Medicine Plastic and Reconstructive Surgery Psychiatry Pulmonology Radiation Oncology Radiography Rheumatology Specialist Physicians Surgery / Paediatric Surgery Urology
19.	South African Network of Art Therapists	083 – 656 4737	Art Therapy
20.	South African Podiatry Association	011 – 622 4680	Podiatry
21.	South African Society of Clinical Technologists	012 – 344 3800	Clinical Technology
22.	South African Speech Therapy and Audiology	011 – 653 2114	Speech Therapy Speech Therapy and Audiology